

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Cc: [May, Alfred](#)
Subject: CP - work update, 02/12/2007-02/23/2007
Date: Monday, February 26, 2007 5:04:43 PM

On my Plate

ED & Asthma epi/surveillance paper
Drinking Water Content Workgroup / Data Resources Sub-team
Arsenic in water, data evaluation and mapping
Birth outcomes and ambient air, multi-state project
Cancer Registry geocoding support
CLPP - surveillance definitions support/discussions
Other indicators for ambient air, birthweight - **Syndromic Surveillance for CO**
IPHis AVR toolkit
CO Surveillance Database
Other MeCDC: Birth defects cluster inquiry

Time Allocation

Task	Target - revised	Reality
ED & asthma epi/surveillance paper	10%	<5%
Drinking Water Content Workgroup	20%	5%
Arsenic in water, data evaluation and mapping	20%	35%
Birth outcomes and ambient air, multi-state project	20%	<5%
Cancer Registry geocoding support	10%	25%
Other indicators for ambient air, birthweight, CO	<5%	20%
IPHis AVR toolkit	<5%	0%
CLPP support	10%	10%
EOHP - hiring, staff support, IT thorns	<5%	<5%
CO Surveillance Database	5%	<5%
Weekly update	<5%	<5%

Activities/developments

ED & asthma epi/surveillance paper
o nothing new here

Still true: I'll need to declare a week (or two) to work on this, to the exclusion of almost everything else.

Drinking Water Content Workgroup / Data Resources Sub-team

- o Water CWG
 - joined Indicator sub-team for Arsenic

Data Evaluation sub-team is awaiting other States' comments on Public Water data survey tool.

Arsenic in water, data evaluation and mapping

- o Lisa has been feeding me sample data reports from StarLIMS
- o explored HETL data for glitches and other potential issues: i.e., missing fields, ambiguous entries, etc.
- o inquired of MEGIS regarding ArcGIS tools for spatial interpolation

Data cleaning is going to be a headache, but we're used to that. Response from John Nims about availability of pre-StarLIMS data was scary ('it's in a file cabinet, on paper'), but probably misleading. Lisa is very close to generating the essential data and attributes essential for analysis.

ArcGIS has a tool for spatial interpolation, by way of an extension, but I learned that I'm the only person to ever ask for it! MEGIS said they can install a 30-day demo for me. I said I'd wait until my data is prepped and ready for mapping.

Birth outcomes and ambient air, multi-state project

- o no new developments

Cancer Registry geocoding support

- o wrote up an extensive brain dump of strategies and issues related to geocoding
- o evaluated address cleaning/conversion software
- o attended EPHT-MCR meeting

I put considerable effort into a lengthy summary regarding potential methods for geocoding, and issues related to census tract assignment and tract-level analysis. A number of my suggestions (e.g., land parcel data and commercial ZIP+4 data) have never been brought up by MEGIS. This was cc'd to Molly, and to Dan Walters. Apparently this memo was of sufficient density to dissuade any response.

I also evaluated the commercial address cleaning tool which CDC has implemented as part of their Lead data cleaning wizard. I recommended to Castine that they license this ASAP and apply it to Registry addresses before MEGIS does a final geocoding run. Of key interest: the tool's database includes E911 address conversions (from e.g. rural routes). In principle, that information could be spotty, but I found a zipcode-level tabulation of the # of address conversions and, happily, every zipcode in Maine was >0.

There was also a late request for input on Portland-area geocoding to aid the follow-up of the North Deering cluster/environmental inquiry. I did some overlays and checked data availability. I believe this is a good training opportunity for MCR to do the geocoding themselves - but of course there needs to be some QC applied.

CLPP surveillance definition and data support

- o no new developments, for my part

Discussions and consultations with Lisa P, per usual.

Other indicators for ambient air, birthweight

- o full day of meetings regarding Syndromic Surveillance pilot project, and use of CDC EARS software tool

This soaked up some time. But I'm favorably impressed with Amy Robbins' and Don Ward's handling of this project, and by the reasonable, no-nonsense, it-is-what-it-is approach of the CDC staff supporting the analysis Tool. On the downside, it may be a while before some major hospitals in Maine contribute a data stream.

Interestingly, it seems that others are trying out SS for CO poisoning, for instance in Boston and Vermont. It's not clear whether they include very non-specific symptoms as part of the definition, or only mentions of "CO"/"CO2". I'm going to follow up with contacts in the aforementioned places.

My initial take stands however, that the Tool's utility for us will be to compare data coming from the hospital ED logs to the reports we get through the formal CO surveillance system.

IPHS AVR toolkit

- o nothin'

EOHP - hiring, staff support, IT thorns

- o no developments regarding Lead Epidemiologist candidates
- o I applied for a USM job

I snail-mailed a CV and cover letter to Doug last Wednesday, and emailed him the same later. I haven't heard back.

CO Surveillance Database

- o no developments

Other MeCDC

- o birth defects cluster inquiry response

Meeting and correspondence from Kathy Decker. Meeting included Ellie M.

There is a practice/nurse query regarding birth defects in Vassalboro or surrounding area. As would be expected, the number of unvalidated defects in Childlink are either similar to statewide rates, or marginally of interest (i.e., excess), depending on the time/place cutpoints. At this point in time, the numbers are too small to follow up on. Kathy found excellent materials and protocol for birth defects cluster investigation from California. I directed her to MCR, for letter templates and protocol for "heightened surveillance" in this kind of situation. This inquiry could fizzle, or it could become more interesting - in a year's time.

I don't understand the current limbo status of Birth Defects right now, but supposedly there is a

new hire coming on imminently?

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, 07/30/2007 - 08/24/2007
Date: Friday, August 24, 2007 5:30:02 PM

On my Plate

EPHT

Asthma/air surveillance

- revise JPHMP article
- plan next stage of analyses
- follow-up with DEP
- respond to Asthma Program's request for material for Burden Document

Water

- arsenic in private wells mapping project
- Content workgroup

Lead

- start geocoding EBLs for 2005-06, then do screeners
- assist with indicator piloting
- aggregate census tracts to towns for reporting to Portland Public Health
- moral support for database issues, and reportable #s comparison with CDC

Cancer

- develop address cleaning and geocoding protocol
- train MCR staff to manipulate, geocode, and resolve data for geocoding

Birth outcomes

- multi-state collaborative
- geocoding

Geocoding, general issues and tools

- procure data/software

Other

- meet with Tracy Kolian

Program/MeCDC

Cancer Inquiry Team

- meeting to discuss public queries, evaluate data, and respond

IPHis

- AVR Toolkit demos and evaluation

IT planning

- AD migration lists, discussions, testing
- procuring a Wiki for testing as project/program management tool

These week's activities...

EPHT

Asthma/air surveillance

The JPHMP article is DONE! (until reviewer revisions...)

Responded to DEP, never heard anything back. Andy and CP discussed and concluded that sub-area analysis not likely to illuminate anymore, based on DEP findings of region-wide ozone in 2003.

Read Asthma Burden Document developed by CSTE Fellow; MeCDC epidemiologists had requested EPHT contribution or "environmental trigger" data. The Document is a good resource as a compilation of many Maine BRFSS factoids about asthma prevalence and the characteristics of people with asthma in Maine. Overall, it's quite underwhelming though. There is no narrative about asthma in Maine! It's just endless bullet points and bar charts, with detailed tables in the appendices. The only text is an introduction which talks about asthma generally and nationwide. But nothing particularly insightful about asthma in Maine. Given all of the above, plus the fact that Katie requested only data on environmental triggers, and the fact that the case-crossover results on hand are pending publication, I didn't give them any of that material to include.

For our contribution, I gave them the updated Healthy Maine 2010 materials related to ozone. I also commented on secondhand smoke, particulate matter, and pollen, but advised that the only essential item to include was the ozone stuff from the HM2010 update.

Water

Attended 2-day conference in DC, via telephone. The first day I learned a lot about DBP, SDWIS, and methods for dealing with censored data (i.e., non-detects). The second day was digressive. Indicator packets need to be developed by 9/15. I volunteered to help wrap up the Arsenic in PWS one.

Right now there simply isn't much constituency for private well-related EPHT Indicators. SDWIS provides a rough'n'ready source applicable to much larger populations in the current crop of EPHT states. As far as indicators/data for the National EPHTN, it's probably a post-2008 goal. Need to follow up on groundwater/private well data survey.

Arsenic in private wells - geocoding and mapping

I have cleaned the addresses and filtered out a lot of chaff. This project has stalled with three impending tasks to go: 1) attempt to de-duplicate same-well results (pick first test); 2) geocode the addresses, resolving the tricky ones as possible; and 3) mapping as point plots as well as by town summary measure. That will satisfy our Year 1 goal. But it leaves much left to do, such as a) create a smoothed interpolation, b) obtain legacy data from HETL (we may already have it), c) obtain data from private testers, and d) pilot test a linkage with e.g., birth records.

Lead

Another geocoding task. EBLs should be easy, 'cause the numbers are small. But it turns out that the original address submitted with an elevated sample may be out-of-date, or otherwise not reflect the "address of exposure." Linking to the Case management system allows identifying other addresses, but which is the appropriate one to represent the child? This turns out to be tricky, and the best/only way to determine an answer is for Tina/Sue to look at the case records and figure it out. This also turned into a 12-15 hour project for MCLPPP, at a time when Sue is gone, causing some consternation.

Hopefully this will feed into an improved database design where it's easy to code and retrieve the "case-causing address".

After having MaryAnn and Kathy look it over, I reported a town-aggregated table of 2005 EBLs to Portland Public Health. Everything but Portland and Westbrook were censored.

I ran 100,000+ testee addresses through the address standardizer. Learned some things. For example, the County output can be erroneous, and the lab doesn't include State for the address field(!).

Interactions with CDC continue to confound.

Cancer

See Geocoding tools task, and Cluster Inquiry Team

Birth Outcomes

Requested that my name *not* be included as author on collaborative paper.
Referred Dan to Kathy about making data request for birth records.

Geocoding, generally

Obtained address validation software (thanks, MCR). Rhonda received invoice for Zip+4 and Zip+2 data order. One remaining tool to purchase: statewide supplemental address source.

On a regular basis it becomes clear that geocoded data is the key that unlocks a lot of analytic capabilities and makes many problems vanish. Like the problem of assigning addresses to Zip Code Tabulation Areas based on USPS zipcodes. Not a problem if the addresses are geocoded to streets. Or the problem of responding to Cluster Inquiries with area-based rates. If the data were geocoded, most of the work would be done prior to an inquiry.

Other

Met with Tracy Kolian. Clearly not a scientist, as she noted. But clearly an excellent communicator, with policy skills. Could be a good manager and risk communicator/stakeholder liason/legislative promoter. She can advocate well for EPHT, even while harboring healthy skepticism. Believes in accountability benefit of Tracking.

Program/MeCDC

Cancer Inquiry Team

Discussions surrounding North Deering (Portland), and Castine inquiries, reinforced thoughts about needing generalized, statewide analysis based on geocoded data...

IPHIS

AVR toolkits. Lots of time taken up by demos. I was left with dilemma between Business Objects as a better, more approachable data processing tool, while SAS BI is extensible to sophisticated analytics. Oracle looked to be well-supported, with the same data processing bent as Business Objects, but less approachable.

IT Planning

AD preparation. Tendered list of burning issues/questions to Jeff, who is following up swiftly. One burning issue (with ripple effect) was how the on-site application server could or could not be integrated with our S:hared directory structure. A list of hardware and software to be dealt with is nearly complete, and we have a testing machine in place. We have not run those by Jeff yet. Most of the items on your To Do list have been dealt with - either resolved, or inventoried in a list. The only item which hasn't been fleshed out is the server hierarchy and permissions, which partly depends on Jeff/OIT's solution to the question about integrating separate servers (on-site and not) into a single S:hared directory structure. The workbook of lists is here:

G:\Admin\Active_Directory\Software_Hardware_AccessMDB_list.xls

Eric and I met with Paul Sandlin (eGov services) about procuring a web-based Wiki for workgroup and project management. I was impressed with his approach and competency. To date though, he has not convinced OIT of seeding the project, so it would probably be provided with a fee, and when they get around to it. Earlier, he said the standard application fee would apply - \$250/month - but that could be shared among however many programs want to use it. He had hoped to forestall this cost for a year's worth of seeding/pilot testing. We've tagged the DWP as potential early-adopters.

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, week ending 09/07/2007
Date: Monday, September 10, 2007 9:45:59 AM

On my Plate

EPHT

Asthma/air surveillance

- test and comment on Air indicators
- plan next stage of analyses
- peer review of other case-crossover study

Water

- arsenic in private wells mapping project
- Content workgroup:
 - o review of Indicators templates
 - o pilot test Arsenic in CWS indicators

Lead

- start geocoding EBLs for 2005-06, then do screeners
- assist with indicator piloting
- moral support for database issues, and reportable #s comparison with CDC

Cancer

- develop address cleaning and geocoding protocol
- train MCR staff to manipulate, geocode, and resolve data for geocoding

Birth outcomes

- multi-state collaborative
- geocoding

Geocoding, general issues and tools

- procure data/software

Other

Program/MeCDC

Arsenic Exposure Study

- check out BMDS software
- start writing up methods
- do some analysis

Cancer Inquiry Team

- meetings to discuss public queries, evaluate data, and respond

IPHIS

- AVR Toolkit demos and evaluation

IT planning

- AD migration lists, discussions, testing
- procuring a Wiki for testing as project/program management tool

USM

see **Note** below about faculty meetings!

This week's activities...

EPHT

Asthma/air surveillance

I finished the pilot testing, making maps, and sending comments to Tom and Fred. This took longer than expected.

Water

Reviewed penultimate draft Indicator for arsenic in community water supplies. My recommendation to change "drinking water" to "tap water" won't fly - it's ahead of its time.

Arsenic in private wells - geocoding and mapping

No progress.

Lead

Some continued consultations with Kathy regarding the converted addresses and the work she is doing to resolve differences between the original and "cleaned" versions.

I reviewed a marked up list of addresses for the elevated lead test results (2005/2006) from Tina, and had a discussion of the markup and some of the curious results.

General

Read annual report documentation and attended group meeting.

Note: uh-oh, Doug has scheduled biweekly faculty meetings on Tuesdays, 11-12, starting 9/18.

Cancer

Set a meeting to assist with geocoding Portland data (in support of North Deering

investigation).

Birth Outcomes

Nothing this week.

Geocoding, generally

Did paperwork (license agreement, etc.) to obtain Zip+4 and Zip+2 data.

The one remaining tool to acquire, budget permitting, is the pro-quality street data (~\$3,200/yr).

Other

Nothing.

Program/McCDC

Cancer Inquiry Team

Nothing this week.

IPHIS

Nothing this week (for me).

IT Planning

AD preparation.

- to-and-fro emails with Jeff, testing of links to servers
- some software testing (Tina and Eric-specific)

Time spent:

- <5% - Arsenic in well water mapping project
- 15% - EPHT general stuff
- 15% - Lead address cleaning
- 5% - Water indicators template, conference call
- 40% - Air indicators: piloting, discussion
- 15% - AD migration testing, discussions
- <5% - this update

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, week ending 11/21/2008
Date: Friday, November 21, 2008 4:49:43 PM

On my Plate

- EPHT – Portal development
- Lead maps, reporting
 - Water CWG – data, conference calls
 - Enterprise Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project

Lead - address cleaning/geocoding, 2003-2007

Cancer – North Deering inquiry
- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – Meetings, misc.

This week's activities

EPHT

Portal development - Data acquisition/compilation/display creation:

- Finished maps of “high density” areas pre-1950 housing, by Census Block Groups
 - look pretty good
 - learned how to turn off green layers of basemap
- computed town-level pre-1950 housing for 5 areas, based on aggregating Block Group data
 - added this to summary data, which Lisa will upload to Cognos and use to create Tables and Charts
- Town-level pre-1950 data for all towns? *This would be good to request from Courtney – looks like she's generated the data, though the map you showed me was labeled “1950-59”.*
- *change basemap color scheme for EBLL and screening maps?* I could try it and see what you think.

Portal development – *thoughts on content format*. The high density area maps are really pushing the viability of *image-as-content* to the limit. Resolution and legibility are lost in the maps. Meanwhile, we have “table” output that can be viewed - but not copied as text and numbers. Embedding Cognos will take care of most of this, but not the ArcGIS-generated maps. To workaroud these issues, will we push the scope, or hack the Portal? For instance, with a little trickery, we can (probably) embed PDFs in the existing system. But should we put this off to January – or even post-February? Wait for user feedback? Just pondering here.

Portal development – confidence interval sideshow...CIs for EBLI rates for Secure table content. Stata to the rescue. Calculated both binomial and poisson CIs. It was a tad more complicated than I initially thought. Had to create a “do” file with one command per rate, rather than using a field calculation. But still, it took all of 15 minutes to go from input to usable output. Kathy was very grateful. *Aside: we ought to buy the current version of Stata.*

Cognos content – created/re-structured Housing data as above, for use with Cognos. Made specs for Percent of Housing *and* Counts, just to fill out the F-AVR options.

Other mapping – revised statewide town level map with proportional dots (*will send soon*).

Roles

Not yet.

Water Content – haven’t looked at Quick Facts text yet. There is text from the CWG, which I’m guessing is much more (too much more!) detailed than we want.

Air Content– skimmed materials for NCDMs from CDC.

Address Geocoding Tool – no developments.

Asthma/Air Surveillance – no developments.

Birth Outcomes multi-state analysis – no developments. Somewhat related: had contact with Mathew Bampton at USM - GIS guy who is involved in Maine Med research of Cumberland Co. birth certificate data. I’ll talk to him next week and see what they’re up to.

Arsenic in wells mapping project – meeting scheduled for next week.

Other EPHT – GIS Workshop has been deferred to next spring.

Lead

Lead Epi interviews –one in-person candidate.

Support – some mapping done related to Naomi’s Midcoast query.

Cancer

North Deering inquiry – no developments.

Geocoding protocol – no developments.

CO surveillance

Analysis contract – No devs.

Program

Arsenic Exposure Study – no developments.

MHDO data issues – No devs

Wiki – they configured it so we can link to files on the server. But now the general functionality of external links is broken! The veracity of this thing, I think, will depend on how vested the OIT progenitors are in using it for their own purposes (i.e., they'll fix it if it impacts their own use).

ME-CDC

Data Workgroup – no devs from my vantage.

USM

Meetings, goings-on – upcoming AMS meeting with USM President, 12/2. Can't miss. Need to do some background checks on the Prez. Supposedly she was involved in creating a school of public health.

Deadlines

Demo Maps for Portal – finish Lead Environmental (=Housing age) by **11/26**.

- Selected area maps are done; will need to review and QC Cognos reports that Lisa generates next week.

Text, reports, etc. for Water– mid-December (?)

Text, reports, etc. for Air– mid-January (?)

Time spent:

65% - EPHT Portal-related (includes Lead)
5% - Lead support/data request (maps for Naomi)
20% - Lead Epi candidate evaluation/interviews
<5% - Water CWG: data generation, conference calls
<5% - Geocoding Tool development
10% - ME-CDC, MHDO, etc. meetings
<5% - this update
<5% - USM meetings

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 03/20/2009
Date: Friday, March 20, 2009 4:37:46 PM

On my Plate

EPHT – Cognos AVR development

- Portal development
- Glossary/text
- Security issues
- Water CWG
- Enterprise Address Geocoding Tool
- Asthma/air surveillance
- Arsenic in wells mapping project
- Birth outcomes multi-state project

Lead - update, re-sync of geocoded/cleaned address data

Cancer – North Deering inquiry
- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup
- Epi Resource group

USM – Meetings, misc.

This week's activities

EPHT

Portal development - Data display creation:

- Newfangled tabbed view with interactive output displays
- Created in last two weeks: Charts, and Maps (with dual comparison view)
- Added age-group option to Map tool (see current Tabbed portlet)
 - Potential problems with data scales as data are sliced, and with absolute measures like raw number of hospitalizations
- Need to re-visit Charts and Trend Charts – think about how/if to add age group “by” option – or split this off to alternate display
- Need to add interactive Table option – should be easy, compared to Maps!...famous last words.

Portal development –Security. Cognos has not been vetted, and I’d say we’re in the process of having that done. The dangers are not so much exposure of any sensitive data, as potential damage to our work and embarrassment to State and loss of public trust.

Portal sideshow: Helped move static content to Maine.gov with appropriate permissions set.

Roles

Not yet.

Water Content – need to get back to Cognos pages of Water NCDMs someday...

Air Content– have data; one Cognos report to date.

Address Geocoding Tool – no response from OIT regarding ‘production’ status.

Asthma/Air Surveillance – no developments.

Birth Outcomes multi-state analysis – no developments.

Arsenic in wells mapping project – received Excel spreadsheet, forwarded to Eric; received paper files (maps) from Martha.

Other EPHT – GIS/Google Earth training, early May in Albany? Have to ask Tom T. what the dates are.

Lead

Support – occasional requests regarding maps already produced. Kathy mentioned the need to update our geographic/geocoded surveillance database with 2008 data, organize it and hand it off to Shuqin.

Cancer

North Deering inquiry – report from Kathy, some queries from Molly. Haven’t looked at it yet.

Geocoding protocol – no developments.

CO surveillance

Issues with ecologic analysis – Follow-up meeting with Doug, not scheduled yet.

Program

Arsenic Exposure Study – no developments.

MHDO data issues – Wondering about asthma data for 2003.

ME-CDC

Data Workgroup – no devs from my vantage.

Epi Resource workgroup – volunteered to initiated Wiki page for Epi's to share information and create inventory of skills/know-how.

USM

Meetings, goings-on – nothing new.

Deadlines

Demo Maps for Portal – ‘complete’ demo of new Asthma Portlet by 3/31

Portal, portal, portal

- move on to CO, other topics;
- wrap-up water pages
- follow-up on Cognos security issues

Time spent:

- 120% - EPHT Portal-related
- <5% - Lead support/data request (maps for Naomi)
- <5% - Water CWG: data generation, conference calls
- <5% - Geocoding Tool development
- <5% - ME-CDC meetings
- <5% - this update
- <5% - USM meetings

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 03/21/2008...and 06/20/2008
Date: Monday, June 23, 2008 2:57:30 PM

[NB: the last draft of a report from me had a date of 03/21; this includes that material after the more current stuff.]

EPHT

Portal development

- think, comment, discuss, research...
 - content, design, and interface components
- generate (Lead) maps for display
 - generate clean data necessary for reports/maps!
- learn/experiment with Cognos Reporting environment
 - how to integrate with EPHT Portal
 - install Maine boundary maps (counties, public health districts) in Cognos for map reports
 - [Note: requires Mike Biddle to convert/install what I create]

Asthma/air surveillance

- 2006...
- Update asthma-ED/ozone case-crossover analyses with data from 2004 and 2005 and
 - participate in Asthma 1-day conference
 - review/comment on Baltimore Workshop Final Report, and Talbot et al. white paper
 - get better acquainted with Case-Crossover Analysis Tool

Water

- ~~draft proposal to assist PUC in creating GIS layer of CWS service area boundaries~~
- CWG:
 - o run updated NCDM code as necessary, and to evaluate data displays
- draft/procure Data Sharing Agreement with DWP
 - o discuss with Roger, Robin
- arsenic in private wells mapping project
 - o ~~provide summary of efforts for interim report~~
 - o convert to student project?

Lead

- ~~making maps of '05-'06 data~~
- ~~assist with indicator piloting~~
- assist with address cleaning/resolution of all Lead addresses
- assist with data requests – e.g., census unit tallies for Lewiston-Auburn
- conduct geocoding for all screenees, and '03-'04, '07 EBLs
 - ascertain rental status for all screenees, '05-'06
 - o **say what?** How about just the EBLs statewide – and this should be a “for

later” task

Cancer

- ~~train MCR staff to manipulate, geocode, and resolve data for geocoding~~
 - o ~~install and orient to ZP4 address standardization tool~~
- develop address cleaning and geocoding protocol
- North Deering Project – geocoding, census unit assignment, report assistance
- see below for more MCR-related work

Birth outcomes

- multi-state collaborative
- geocoding

Enterprise Geocoding Service

- procure data/software
- edit DOU, project tasks

Metadata

- ~~continuing input on metadata portal tool, content development~~

Program/MeCDC

Arsenic Exposure Study

- ~~give talk at USM seminar~~
- assist Erin with access to files, old email, etc.
- check out BMDS software
- do more analysis

Cancer Registry

- ~~data summaries for MHHGH Cancer GIS~~
 - o lung ca counts by town or region [**DONE**]
- North Deering Project – geocoding, census unit assignment, report assistance

IPHis / Data Access workgroup(s)

- attend Data Access sub-group meetings

IT planning

- procure a Wiki for testing as project/program management tool

USM

- web site bio
 - faculty meetings
- whatever comes up: teaching?

Narrative

Portal and related things: I've had my head deeply wrapped in Portal-related issues - like most everyone else. I have spent a lot of time learning and futzing with Cognos, to explore the gradations with which we could integrate it (or not) with our custom Portal. My first assessment ("not") was off the mark, and I committed an error of process: I should have phrased the shortcomings I saw in the form of questions to Mike Biddle and/or Cognos Support. There are indeed workable ways of loosely integrating Cognos Reports; however, I still have security concerns, and doubts about whether there will be an installation accessible from outside our firewall in the next couple months. My sense is still that Cognos is not a great product, and not the best for ME-CDC users, but we should certainly milk it for all it's worth.

I have spent untold hours researching "client-side" tools for displaying tables and maps. The benefit of these sorts of solutions is that they don't involve programming (per se), engage the user in interactivity, and have applications outside the Portal (e.g., sharing GIS data with staff and collaborators). However, it's pretty clear that our Portal will not allow much leeway in terms of customizing content, at least at this stage: we will have a relatively few pages hard-coded into the JSP programming, and otherwise our control over content will be limited to what we can do in PDFs that are hosted in the Repository. At one point I had the impression that we could put whatever and however much web content we wanted to below the header/banner. That's not the picture I have now.

I hope our Portal can evolve toward this conception of a content management system (CMS): "A [Web content management system](http://en.wikipedia.org/wiki/Content_management_system) is a CMS designed to simplify the publication of [Web content](#) to Web sites, in particular allowing content creators to submit content without requiring technical knowledge of [HTML](#) or the [uploading](#) of files." http://en.wikipedia.org/wiki/Content_management_system

At present, I would say that the Portal design, from the authorship perspective, is more like a *file* management system: it manages the access and presentation of (PDF) files. But it complicates the presentation of web content (admittedly, this is partly a consequence of delivering content customized to particular users/roles).

Cognos: I spent two days' training, and numerous hours experimenting with Cognos. I met with Mike Biddle, Rene, and Lisa. There are clearly some issues for which I can be an alternative resource to Lisa, rather than Mike B. I learned how to generate map files to be transferred into Cognos, and gave Mike B. a county map file to use as a trial. However, this is something that requires Mike's system-level access to implement, and I have not had a response from him following through. If we want to integrate maps with Cognos Reports, this is the best way to go. If we're happier keeping maps separate, then it's not a high priority.

Lead data for the Portal: numerous meetings on the array of tabular and chart content we want to present. We have touched on the types of maps we want to present, as well, which will be closely related variations of maps already created and presented to the Lead Advisory groups. However, if we really want strong data-driven decision making, *we need to clean up 5 years worth of data (2 down, 3 to go)*. What needs to be done? Complete the general address clean-up process, geocode them, and go through the task of assessing "exposure address" for the EBLs. This is a couple weeks for me, and would involve several days of part-time interaction/work with Tina and Lisa.

Lead data (other): I'm working on census unit tallies for a Lewiston-Auburn request (akin to Portland/Cumberland Co.). This will be so much easier (and better) when we can share actual addresses with them! They'll be able to choose the census units that fit their aims and (hopefully) make sense of the point clustering.

We should be thinking about the "terms" and technologies under which we transfer address data to them. I can imagine some people will want GIS files of the EBL addresses - and maybe that's fine, as long as we remove year-specific age and personal identifiers. But what if they want to re-release or present the data? Should the terms be: it must follow ME-CDC privacy policy, and have prior approval from ME-CDC? What are the FOIA boundaries here, given the new statute?

Asthma/Air analysis and related goings-on: the Asthma Event (already reported on). Tom Matte and Aaron Cohen sent out a draft report from the Baltimore Workshop, which I'll need to review by the end of the month.

On the docket: to share/discuss BRFSS results with DEP and others. I believe AI noted somebody from WI presenting data on this topic at CSTE.

Also: Ruth L-S will want me to participate in an Asthma workgroup which started brainstorming at the Event.

Water – Data Sharing Agreement, and NCDMs. The draft TPA was fine with Roger, and he's ready to sign the dotted line. I told him I'd have you look at the draft before having him sign. I added Lead Exceedances to the data request, for posterity (this is not part of the initial NCDM data submission). CWG: There have been some updates to the NCDM code, mostly related to graphic display output. At some point, I need to update our SAS code, and review the display output for material that could be used on the Portal.

Cancer Registry: I qc'd the MCR staff's efforts at geocoding Portland area Registry addresses, then assigned census tract IDs to the Cases. This is for the sake of a North Deering (Portland) inquiry, that Castine could not finish before she left, and which Kathy is now working to complete (after hours). This census tract-level analysis is precisely the kind of investigation that will be much more efficient when their data is completely geocoded; even more so if it was integrated with a tool like RIF. The more generic task I've been assigned is to develop a geocoding protocol for them. There is a draft protocol, but finishing that document will be tied to a) my hands-on work with Lead addresses, and b) what comes out of the Enterprise Geocoding Service.

Enterprise Geocoding Service: a nearly signed DOU. I expect I'll need to solicit weekly (if not bi-weekly) progress updates from ADAM. I expect the first iteration to be a hardly-changed update of the prototype they showed earlier. However, if they can deftly handle all the features I put in the DOU, the service will be useful and should garner support for further development. Another item of progress is that we've purchased a license for NAVTEQ data. This means I don't have to resort to mickey-mouse techniques for geocoding in towns where E911 data has not been released. Plus, it gives a good cross-check everywhere else.

Water Service Area boundaries (PUC data): preliminary meeting, project specs, etc. – for work to begin next fiscal year.

Arsenic Exposure Study: I transferred Erin's old email to a Google account, so she can access it from wherever (with state of the art search!). No other developments.

Data Access Workgroup: actively participated in one meeting. Looks like there will be a demo next Wednesday, which I probably won't be able to attend.

USM: Doug wants to hand off the Intro Epi course next spring. Thankfully, Katie is enthusiastic about this opportunity to teach, while I remain enthusiastically ambivalent. However, I'd like to do my share (as long as that isn't too much). Will that be a couple lectures? A partial class every other week? I'll have to discuss with Katie.

Misc: Lead Candidate talks and meetings; arsenic test geocoding; minor consultations and epi paper readings.

[from 03/21/2008]

On my Plate

EPHT

Asthma/air surveillance

- participate in EPHT portal development
- pilot test AVR tools on Air data
- Update asthma-ED/ozone case-crossover analyses with 2004 and 2005 data (and 2006?)
- review and provide comments on Air Group indicators

Water

- draft proposal to assist PUC in creating GIS layer of CWS service area boundaries
- CWG:
 - o pilot test Consistent Data and Indicators for:
 - DBPs
 - arsenic
 - o assist with implementation guide revisions for arsenic
- arsenic in private wells mapping project
 - provide summary of efforts for interim report

Lead

- making maps of '05-'06 data
- ascertain rental status for all screeners, '05-'06
- assist with address cleaning/resolution
- make plan for geocoding all screeners
 - assist with indicator piloting

Cancer

- train MCR staff to manipulate, geocode, and resolve data for geocoding
 - o install and orient to ZP4 address standardization tool
- develop address cleaning and geocoding protocol

Birth outcomes

- multi-state collaborative
 - o assist Dan W. in obtaining records for study
- geocoding

Geocoding, general issues and tools

- procure data/software
- spearhead agency-wide service

Metadata

- continuing input on metadata portal tool, content development

Program/MeCDC

Arsenic Exposure Study

- ~~give talk at USM seminar~~
- assist Erin with access to files, old email, etc.
- check out BMDS software
- do more analysis

Cancer Registry

- data summaries for MIHGH Cancer GIS
 - o lung ca counts by town or region
- Cancer Inquiry Team
 - o meetings to discuss public queries, evaluate data, and respond

IPHis / Data Access workgroup(s)

- attend Data Access sub-group meeting

IT planning

- procuring a Wiki for testing as project/program management tool

USM

- web site bio
- faculty meetings
- whatever comes up

This week's activities...

EPHT

Asthma/air surveillance

The manuscript is really done ("in press" I guess).

I edited, discussed and contributed to a joint white paper with Tom, Fred, Valerie, plus Evelyn Talbott and another Pittsburgh person. This was hard. There were some similar points as the ones we made in the manuscript, but they were rather submerged in the white paper. I tried to draw them out.

Preparation needs for the Workshop: a 10-minute (!) piece - slides requested by Friday the 11th. There is a fair amount of background material. I would like to go with a fairly clear idea of the main questions for the group, from our superior perspective.

Air Indicators: unless I'm missing something, you've already voted and provided comments on these. I reviewed the last revision of the Indicators, which altered the PM ones to better fit the model set by the ozone measures.

No other developments related to Asthma/Air.

Water

Two hot projects here: 1) piloting SDWIS data extraction and deriving Data and Measures; 2) supporting the creation of water service area boundaries with PUC/OIT.

I contacted PUC staff to ask about the Water District GIS project. Here's what I've heard, and expect to hear more details early next week:

"The work was begun when we thought our bigger project had received the green light in March. Then it stalled when/while OIT reorganized and worked through whether or not it would support new ArcIMS projects or force agencies to use ArcServer which until just recently it could not support. When I'm back in the office on Wednesday I'll talk to Joe to see if we can figure out how to give you access to our test data to see if you can use it. It is all public data so there are no security issues – some of it is water district data."

Arsenic in private wells - geocoding and mapping

No new developments.

One thought, re: "I think the biggest use is in promoting testing, ...yes everyone should test, but we may be able to get communities to put more efforts into getting their residents to test if we can show them they are in a higher risk area."

Can we promote testing in some areas without the side effect of *demoting* testing in other areas? My answer: yes, so long as the message is targeted/limited to precisely those communities we want to encourage, rather than to everyone in the state.

Lead

Cancer

MIHGH: Lung cancer #s by town for northern Maine.

Received data from Castine, and started working through the process of standardizing town names to MEGIS conventions. But then Castine said she was going to expand the original selection of data – so I'm waiting for an update.

Birth Outcomes

Dan W. contacted us about submitting application material for birth records. Kathy sent him requisite documents and instructions.

Geocoding, generally

Enterprise Address Standardization and Geocoding Service

Other

USM bi-weekly meetings.

Program/MeCDC

Arsenic Exposure Study

Cancer Inquiry Team

North Deering Project.

Need to help MCR finish off geocoding and census-level aggregation of Portland/South Portland cases – before Castine leaves.

AVR Toolkit & Cognos Testing

IPHS & Data Access Workgroup

Other IT-related stuff

Plotter resurrection.

Time spent:

<5% - Arsenic in well water mapping project

45% - EPHT stuff: water workgroup

25% - Lead Support/Lead address geocoding project

<5% - Air indicators: piloting, discussion

15% - Geocoding support, general protocol developments

<5% - Arsenic exposure study

<5% - this update

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 05/01/2009
Date: Friday, May 1, 2009 5:05:11 PM

On my Plate

EPHT – Cognos AVR development

- Portal development
- Glossary/text
- Security issues
- Water CWG
- Enterprise Address Geocoding Tool
- Asthma/air surveillance
- Arsenic in wells mapping project
- Birth outcomes multi-state project

Lead - update, re-sync of geocoded/cleaned address data

Cancer – North Deering inquiry
- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup
- Epi Resource group

USM – Meetings, misc.

These week's activities

EPHT

Portal development - Data display creation:

- Re-jiggered informational “About” pages for Asthma, both now hosted by Cognos
 - features pop-up links to Contact and Glossary pages
 - About the Data page has internal link (aka bookmark) to skip from top to Technical Info section
 - Problem: “static” content served by Maine.gov has automatic maine.gov header – which has Portal-unrelated items and will only confuse our users. We need to find a way to disable this or else use another host.
- Asthma page feedback
 - Good comments/observations from Eric, raising some tricky

issues

- No comments from any other beta tester
- Spec'ing CO tabbed page
 - Meeting with KD and LP
 - Pretty straightforward, data scope is manageable
 - One curiosity is mixing in BRFSS CO-detector data: nice to include for comparison, but it's different in terms of units (=household) and source (survey).
- Lisa noted an odd output with the Asthma Chart tool:
 - If a user selects a checkbox for gender but does not click 'Next', and then picks another prompt option...it's possible to get a display with a misnomer title.
 - I'm not sure if this will matter. It depends on how methodical the users are. There are workarounds...

Portal development –Cognos security. Found another backdoor into Cognos, quite by accident. It's not clear if it allows the anonymous user to enact any damage, but it does look like a way to see things that are supposed to be hidden. It also would allow a logged in user to see things they shouldn't see, and perhaps execute actions upon likewise.

Portal sideshow, Cognos reliability: Last weekend's outage is blamed on a faulty web cache, not related to Cognos. The OIT guy, Jason Tourtelotte, says it would usually be cleared by reloading the page. I'm pretty sure that didn't work last Sunday though!

Roles

Not yet.

Water Content – need to get back to Cognos pages of Water NCDMs someday...

Air Content– have data; one Cognos report to date.

Address Geocoding Tool – Mike F. sent a "Production" URL. To me, it looks like it still suffers from verbiage and cosmetic issues that I raised in several meetings. The last prior email from Mike seemed to check off a list of to-do's though. I'll have to revisit that. I updated the Geocoding URL per Rene's instructions. Someday it will show up in the Prod Portal.

Asthma/Air Surveillance – no developments.

Birth Outcomes multi-state analysis – no developments.

Arsenic in wells mapping project – no devs.

Other EPHT – GIS Day at Albany is next Tuesday, with training day on Wednesday (Google Earth and SAS spatial aggregation tool). I'm still working on logistics of attending either or both days.

Lead

Support – met with KD and SX to discuss updating Lead surveillance data, geocoding, etc. Also created spreadsheet of town-level summary numbers for Joe, 2003-2007.

Cancer

North Deering inquiry – report from Kathy, some queries from Molly. Still haven't looked at it yet.

Geocoding protocol – no developments.

CO surveillance

Issues with ecologic analysis – Follow-up meeting with Doug, not scheduled yet. Asked for Doug's presentation form EPHT Conference, never received.

Program

Arsenic Exposure Study – no developments.

MHDO data issues – Wondering about asthma data for 2003.

ME-CDC

Data Workgroup – no devs from my vantage.

Epi Resource workgroup – created draft wiki page for Epi group.

USM

Meetings, goings-on – nothing new.

Deadlines

Demo Portal – 'complete' demo of new Asthma Portlet [Done, basically]

Portal, portal, portal

- move on to CO, other topics;
- follow-up on Cognos security issues

Time spent:

- 85% - EPHT Portal-related
- 10% - Lead support/data request
- <5% - Water CWG: data generation, conference calls
- <5% - Geocoding Tool development
- <5% - ME-CDC meetings
- <5% - this update
- <5% - USM meetings

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 07/11/2008
Date: Monday, July 14, 2008 1:03:19 PM

On my Plate

EPHT – Portal development

- Lead maps, reporting
 - Water CWG – data, conference calls, sharing agreement
 - Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project

Lead - data requests (Auburn-Lewiston HUD proposal)

- address cleaning/geocoding, 2003-2007
- Battelle risk prediction project

Cancer – North Deering inquiry

- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – misc.

This week's activities

EPHT

Portal development – lots of discussions. Some assistance/experimentation with Cognos. Bottom line for Cognos maps: Lisa can join data successfully; number of categories, colors and cutpoints are customizable, but with none of the automatic aids of ArcGIS, nor the symbology options (scaled dots, dot density). I don't believe there is any layering or zooming functionality, and there certainly isn't any capability to display an ad hoc set of latitude/longitude points. Right now we have county map display only. I need to generate an import file for PH Districts, towns, etc. We should make a list of display layers; it would be best to do this before my 30 day MapInfo demo expires.

Lead maps, reports – meeting with Kathy, Naomi. I have a summary of maps to be presented. I will forward it to the four of us for review. (See Lead topic below for

discussion of address cleanup and geocoding).

Water CWG – data sharing agreement with DWP will be signed shortly. Revised NCDM code was implemented and XML data files generated. It seemed to work smoothly, and XML validated. Prior to submission I will attend a conference call about sensible QC steps, and will explore the data for errors (original and calculation glitches). Final submission slated for Thursday, 7/17. *Metadata* – I’m not planning to update metadata yet, based on a chat with Rish V. last week. There was some conflicting information from SND though (via Lisa).

Address Geocoding Tool – I have tested a single-address web interface, and provided comments on a 0.0.1 version. I have a meeting today (7/14) with Mike F. to discuss (I preferred email, but he insisted on face-to-face). I think the Tool sucks considerably less than it used to. They have added an option to use NAVTEQ data, and it reports back to the user the “nearest match” when a suitable match is not found. However, much of the verbiage is still bogus, and Lisa and Cindy found it to be pretty lame. It concerns me that my opinion is less harsh: it could be that my expectations of OIT work have hit the basement level.

Asthma/Air Surveillance – no developments

Birth Outcomes multi-state analysis – no news

Arsenic in wells mapping project – Charlie Culbertson will take this over. Need to summarize what I’ve done, outline what he can/should do, prior to meeting.

Lead

Data request – Auburn-Lewiston data request for HUD grant. Tasks: devising census-unit based target area based on City request and inspection of EBL locations (2005-2006); geocoding of *screenees*, since HUD requests # of children tested and <10 children in target area and Auburn-Lewiston as a whole. There were ~2,000 screening-related addresses. Dual-source geocoding reduced the manual inspection load to about 5% (100 addresses).

Address cleaning/geocoding, general data QC – lots of discussion in Portal meeting; reviewed species of address validation “errors” (by direct inspection, and Lisa’s summary). I have some general observations on this – not yet circulated – and will take the lead to make sure it gets done (with assistance from Cindy, May, whoever).

Battelle risk prediction project – attended webinar. Generally impressed that the people working on this project are intelligent, capable, honest (“we’ll take whatever data we can get our grubby hands on” – I appreciated that line). They gave us a pre-print article (haven’t read it thoroughly). They’re project is all about “risk prediction” and I have reservations about this endeavor - as in, “what’s the point?” They talked about using the output “predicted risk” as a tool of intervention. But if the predicted risk far exceeds the observed EBLs, what does that tell us? Or vice-versa? Perhaps this works better in Massachusetts where they have extensive building inspection data.

Cancer

North Deering inquiry – No developments really, but I know that I'll be needed to check and review what Kathy does, perhaps generate some maps, etc., and this is a pretty big little project (I think).

Geocoding protocol – I consider my work with Lead addresses and with the Address Tool as all feeding into a geocoding protocol that Cancer will use.

Program

Arsenic Exposure Study – No developments. It would be nice to schedule a future time to work on this.

ME-CDC

Data Workgroup – You were there. Meeting + trial experimentation with the demo Data Access site. I'm still not sure what to make of it in general, but it strikes me as a different approach to some similar information access objectives as the EPHT Portal. So it has complementary advantages and disadvantages. Our groups can learn from each other's efforts.

USM

Meetings, goings-on – No developments (it's summer), aside from travel budgeting and Overhead bargaining.

Time spent:

- 40% - EPHT Portal-related
- 35% - Lead support/data request
- 15% - Water CWG: data generation, conference calls
- 5% - Geocoding Tool development
- <5% - ME-CDC Data Access Workgroup
- <5% - this update

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 08/22/2008
Date: Monday, August 25, 2008 5:10:42 PM

On my Plate

EPHT – Portal development

- Lead maps, reporting
 - Water CWG – data, conference calls, sharing agreement
 - Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project

Lead - ~~data requests (Auburn-Lewiston HUD proposal)~~

- address cleaning/geocoding, 2003-2007
- Battelle risk prediction project

Cancer – North Deering inquiry

- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – misc.

These weeks' activities

EPHT

Portal development – lots of discussions. More assistance/experimentation with Cognos. Assisted with addition of new mapping sources (towns etc.). Investigating various options for delivering interactive point maps of addresses that layer on top of a useful street-level basemap. Cognos definitely can't do that.

Met with Toho Soma of Portland, discussed data availability and walked through screen mock-ups of Portal. He was impressed, said that it would be very helpful. He is working on indicators for Cumberland County. One message heard: even though he works for Portland, he needs town-level data for Cumberland County, not just Portland (and Westbrook). The indicators he wants sound like the measures we are pushing for the portal. I thought he might say the point mapping thing is silly, that he'd rather have the raw data. He didn't say that in the context of mapping, but he did ask about obtaining record-level data in order to calculate the indicators he needs. We told him

our first preference is always to compute the necessary measures in house.

Lead maps, reports – generated sample point maps of Sanford, for Naomi. This led to exploring ways to integrate with Cognos.

Water CWG – no developments.

Address Geocoding Tool – More testing of the single address tool. I used it a lot when working on the Lead Addresses. I'm in the process of obtaining an account so that I'll be able to test the batch tool geocoder. This is what I'm most interested in. Had a meeting with David Blocher and others. It seems high time to involve other interested users to get real world feedback to guide its further development. David Blocher is retiring this week by the way – a pity.

Asthma/Air Surveillance – meeting with Katie, Ruth. Developments: indicator list from Katie to work on.

Birth Outcomes multi-state analysis – no news.

Arsenic in wells mapping project – Met with Charlie and others. Will arrange meeting time for next week or two.

Lead

Data request – Sanford data for Naomi. I considered this related to portal development, investigating the options for delivering point maps.

Address cleaning/geocoding, general data QC – major development of fresh geocoded data for 2003-2007, and Kathy/Lisa are using this as the basis for the Portal measures. Further cleaning (by me) is on hold, but I know May is researching her way through a list. (Aside: apparently she's not comfortable asking providers about patient race. Can someone help her with that?). Discussed exact CI computations with Kathy.

Battelle risk prediction project – any further communication?

New Director candidate talks – I've tried to make arrangements to come in this Wednesday but haven't succeeded so far.

Cancer

North Deering inquiry – No developments directly pertaining to me.

Geocoding protocol – Nothing new.

Program

Arsenic Exposure Study – You tell me what's new, and is it time to make a workplan,

and have monthly meetings?

ME-CDC

Data Workgroup – Meeting this Friday that I probably won't be able to attend.

USM

Meetings, goings-on – No developments.

Deadlines

Demo Maps for Portal – 8/29.

Time spent:

- 65% - EPHT Portal-related (includes Lead
- 25% - Lead support/data request
- <5% - Water CWG: data generation, conference calls
- 10% - Geocoding Tool development
- <5% - ME-CDC Data Access Workgroup
- <5% - this update

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 09/12/2008
Date: Sunday, September 14, 2008 9:38:34 PM

On my Plate

- EPHT – Portal development
- Lead maps, reporting
 - Water CWG – data, conference calls, ~~sharing agreement~~
 - Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project
- Lead - address cleaning/geocoding, 2003-2007
- [?] Battelle risk prediction project
- Cancer – North Deering inquiry
- geocoding protocol
- Program – Arsenic exposure study
- ME-CDC – Data Access Workgroup
- USM – misc.

These weeks' activities

EPHT

Portal development

Data acquisition and compilation:

- Block Group boundaries for Maine
- Census Block Group level data (2000) for Population age <6; and Housing units by Year Built
- Block Group assignment of geocoded Lead addresses (2003-2007)

Quick Facts text

Review, workgroup discussion

Roles

Review, comments

Lead maps, reports – comments/discussion on Kathy and Lisa's samples.

Water CWG – Conference calls. Discussion of sample displays for NCDMs, and draft text for national site.

Address Geocoding Tool – introduced the Tool (verbally) at the ME-CDC GIS user's group. Some Infect. Disease epi's seem interested and have timely needs. Still waiting for functional version of the batch interface. Mike F. proved that he can run it "locally" but the web interface sure doesn't work yet. Don't want to recruit beta testers until it does.

Asthma/Air Surveillance – Publication proof: answered queries and read through it for typos and errors.

Birth Outcomes multi-state analysis – no news.

Arsenic in wells mapping project – Met with Martha Nielsen, and assembled data for her to use. Data included:

- Legacy HETL data (1990-2003)
- Recent HETL data (2005-2006)
- MTBE survey data
- Arsenic Bathing Study pre-filter results
- Columbia U. data
- Census well water used data (1990 census)

Note: we discussed that much of the recent HETL data is lacking addresses, and this could be retrieved by digging deeper into the lab database. But for now, we're leaving that aside, pending Martha's assessment of need. If she does a town-by-town analysis, she can use the "sample town" field.

You might want to take a look at a compilation of maps I put together:

<S:\EPHT\EPHI Feasibility\Arsenic in well water\Data for USGS\Arsenic map comparisons.doc>

At the end of this comparison, it seemed to me that it could be useful just to create an updated version of the style of map/analysis that Marc Loiselle did (i.e., combining max values by town, along with the point display).

Other EPHT – planning group for GIS workshop in November at Rutgers. This is the thing that Dan W. is spearheading. I (and others) have sought to make sure that the topics have direct relevance to portal development. Of course it's partly driven by who knows who and who will be invited. Some people have made contacts with Google.org representatives and other interesting folk. Likely topics to be covered: map smoothing; handling confidentiality; cartographic issues; tracking air quality.

Lead

Data request – Portland (Toho). My indirect involvement is that I cleaned up the town ascriptions in the 2003-2007 data, so Kathy's job in producing the requested data will be straightforward.

Address cleaning/geocoding, general data QC – invested time in translating/cleaning town names for the 2003-2007 data. This relates to the ~20% or so of screening addresses that were not street-level geocoded. It means a) converting locality names to actual town names, and b) cleaning up misplings. May's efforts will (eventually) supercede this.

Battelle risk prediction project – letting it drop? Haven't heard anything.

New Director candidate talks – cost me \$75 in child care to hear Sue and Eric's talks – but I'm glad I did!

Cancer

North Deering inquiry – No developments that I've heard.

Geocoding protocol – Nothing new.

Program

Arsenic Exposure Study – No news.

ME-CDC

Data Workgroup – no news.

USM

Meetings, goings-on – Faculty meetings resume next week.

Deadlines

Demo Maps for Portal – 9/18.

Text, reports, etc. for Water– 12/1.

Time spent:

- 75% - EPHT Portal-related (includes Lead)
- 15% - Lead support/data request
- <5% - Water CWG: data generation, conference calls
- <5% - Geocoding Tool development
- <5% - ME-CDC Data Access Workgroup

<5% - this update

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 10/03/2008
Date: Sunday, October 5, 2008 9:17:23 PM

On my Plate

- EPHT – Portal development
- Lead maps, reporting
 - Water CWG – data, conference calls
 - Enterprise Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project

Lead - address cleaning/geocoding, 2003-2007

Cancer – North Deering inquiry
- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – Meetings, misc.

These weeks' activities

EPHT

Portal development

Data acquisition/compilation/display creation:

- Calculated EBL and screening counts by Census Block and Block Group units
- Housing units by Year Built (i.e., pre-1950) and median year built, for Counties and Districts
 - Summary table to Lisa for upload/modeling in Cognos
- Map series of district-level EBL dot maps (Naomi used for presentations; circulated to EOHP staff as well)
- Draft maps of Portland/Westbrook EBLs 2003-2007 by Block Groups and Blocks
- Lost work due to ArcMap file corruption over Juniper network link

Lead maps, reports – comments/discussion on Kathy and Lisa's samples, plus above.

Side show – sample mapping using Google Earth plugin

Quick Facts text

Workgroup discussion

Roles

I have read Rene's draft, but I have not yet provided comments.

Water CWG – Conference call: demo of mockup AVR for national site.

Address Geocoding Tool – did trial run of batch geocoder. Need to get Mike F. (ADAM) to change some things before it goes to beta testing.

Asthma/Air Surveillance – no developments.

Birth Outcomes multi-state analysis – responded to query from Dan W. that he is filling out birth records application.

Arsenic in wells mapping project – no news from Martha Nielsen (hmm).

Other EPHT – planning group for GIS workshop in November at Rutgers.

Lead

Lead Epi interviews – reading application materials, scoring the screening forms, and conducting phone interviews...this is a relatively new time sink of ~5 hours per week (hopefully over soon).

Cancer

North Deering inquiry – No developments that I've heard.

Geocoding protocol – Nothing new.

CO surveillance (NEW)

Analysis contract – Read protocol, discussed with Kathy and Dr. Smith. Will follow-through with phone discussion.

Program

Arsenic Exposure Study – No news.

ME-CDC

Data Workgroup – no news.

USM

Meetings, goings-on – Faculty meeting and All School ASET Meeting (my first). The new President spoke, the Dean spoke, etc. The President asked everyone to introduce themselves, and she noted the two of us from Augusta. A couple ASET people (the Admin, and the Assistant Dean) thought it would be good to do a story about the Augusta-based people for the School newsletter, and also smaller news-y items to put as announcements on their web site. Whatever.

The most important thing out of all this though, was to feel how disgruntled Erika and Katie are with the USM connection (i.e., Doug). Doug asked for people to teach his epi. class next spring. Katie was enthused, and Erika agreed to co-teach. I had some equivocal interest that I express to Katie, long ago. But apparently, Doug has never given them the teaching materials he promised, and they've gotten very little out of him. The structural problem is that the CDC-USM arrangement doesn't really allow for putting that much effort into the USM side of things, and USM doesn't have any carrots to offer. The issue about the Epi class sounds like it could turn into a point of tension. It might also be partly a misunderstanding, that Doug doesn't expect people to do anything but teach his existing material, with the addition of some practical surveillance examples. But clearly there are bigger issues than that. It left me wondering if maybe USM isn't the right "home" for the CDC/Academics?!

Deadlines

Demo Maps for Portal – 10/8.

Text, reports, etc. for Water– 12/1.

Time spent:

- 65% - EPHT Portal-related (includes Lead)
- 10% - Lead support/data request
- 15% - [Lead Epi interviews](#)
- <5% - Water CWG: data generation, conference calls
- <5% - Geocoding Tool development
- 0% - ME-CDC Data Access Workgroup
- <5% - this update
- 5% - USM meetings

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 10/17/2008
Date: Friday, October 17, 2008 6:31:03 PM

On my Plate

- EPHT – Portal development
- Lead maps, reporting
 - Water CWG – data, conference calls
 - Enterprise Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project

Lead - address cleaning/geocoding, 2003-2007

Cancer – North Deering inquiry
- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – Meetings, misc.

These weeks' activities

EPHT

Portal development - Data acquisition/compilation/display creation:

- Draft maps of “high density” area EBLs 2003-2007 by Census Blocks
 - Public, censored “1-5” dots [I circulated a sample of a couple]
 - Secure, graduated dots [haven’t shown these]
- these take a lot of time and effort to create, so I hope they will be valuable to the Portal, and that I’m not, like, barking up the wrong tree.

Cognos content – many comments/discussion on Lisa’s Cognos-driven tables/charts/maps. Tried to figure out problem with PH District label positions (no luck).

The Portal (for real! kinda) – Rene introduced me to the local-install demo; I learned

some valuable things about the nature of the content repository beast behind the scenes and how it all works. This led to some more experiments, and to some test content given to Rene, exploring ways to embed PDFs, Cognos reports, and more into the Portal. Déjà vu all over again, but now it has a basis in the live Portal environment.

Side show – mapping using Google Earth plugin: generated simulated data in order to give realistic demo to others

Mapping alternatives – attended brownbag demo of MapServer application. This is going to be MEGIS' lightweight solution for web-serving of maps, at a cost of \$75/month for basic service, \$150/month for service with custom programming; this is compared to \$300/month for the ArcGIS server environment. They will have a standard template which will include the State basemap (the green topographical thing that I've used). It's mostly about display, with no real query and GIS features. More sophisticated stuff could be done, but it would require custom programming. There are no plans to have a secure environment, but probably one server for "outward-facing" public data, and a separate one for "inward" behind-the-firewall delivery. It's a little difficult for me to see where this service fits, because it's a little like doing Google Maps overlays, except the basemap is worse and less flexible. The benefit of MapServer is having a pre-built maine.gov interface and relatively quick'n'easy deployment of multiple layers and whatnot. MapServer is much faster than the old ArcIMS servers (like the EIS thing we've seen) too. Mike Smith says this open source platform is very popular at universities/colleges, so it may quickly grow in sophistication and capability. It will be interesting to see what MEGIS deploys with it.

Roles

Uh, I still haven't provided comments on Rene's draft document.

Water CWG – no developments.

Address Geocoding Tool – had feedback/development session with Mike F. on interface and functionality for batch interface. As usual, some oddities, and some encouraging things. On the up side, it sounds like it would be no problem for him to generate a KML file from the output, which people could use to immediately map the results with Google Earth (no ArcGIS required!).

Asthma/Air Surveillance – no developments.

Birth Outcomes multi-state analysis – no developments.

Arsenic in wells mapping project – no developments.

Other EPHT – no developments.

Lead

Lead Epi interviews – a couple more applications; no new interviews.

Cancer

North Deering inquiry – Noted some rumblings from last cancer meeting, but I haven't been pulled in yet.

Geocoding protocol – Nothing new.

CO surveillance

Analysis contract – Had phone conference with Lee Friedman, helped to deliver data securely.

Program

Arsenic Exposure Study – Noted Toenail Soaking Study Protocol.

MHDO data issue – Meeting with Cindy Mervis, Lisa, and Kathy. I was impressed that the problem is worth pursuing until its resolved, though at the level of most analyses the impact may be modest. I am also impressed by Lisa and Cindy's attention to data quality as much as I am depressed by MHDO's lack thereof.

ME-CDC

Data Workgroup – no news.

USM

Meetings, goings-on – no activity of late.

Deadlines

Demo Maps for Portal – gave some to Rene.
Text, reports, etc. for Water– 12/1.

Time spent:

- 80% - EPHT Portal-related (includes Lead)
- 5% - Lead support/data request (maps for Naomi)
- 5% - Lead Epi candidate evaluation/interviews
- <5% - Water CWG: data generation, conference calls
- 5% - Geocoding Tool development

5% - ME-CDC, MHDO, etc. meetings

<5% - this update

<5% - USM meetings

From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, weeks ending 11/14/2008
Date: Monday, November 17, 2008 5:00:38 PM

On my Plate

- EPHT – Portal development
- Lead maps, reporting
 - Water CWG – data, conference calls
 - Enterprise Address Geocoding Tool
 - Asthma/air surveillance
 - Cognos assistance
 - Arsenic in wells mapping project
 - Birth outcomes multi-state project

Lead - address cleaning/geocoding, 2003-2007

Cancer – North Deering inquiry
- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – Meetings, misc.

These weeks' activities

EPHT

Portal development - Data acquisition/compilation/display creation:

- Draft maps of “high density” area pre-1950 housing by Census Block Groups
 - pretty straightforward
 - made to mimic/match all the other high density maps, at least in two-map format and geographic coverage
- as we discussed, this kind of begs for the ability to do layer overlays – e.g., EBLL point maps – for the sake of data exploration.
- Note: “Secure” maps for EBLs have not been finalized. These would be the same format as existing, but with uncensored counts.
Finish for Portal first release?

Cognos content – pursued problem with label placement on PHD maps. Fixed, with help from Will Mitchell, GIS consultant, who will be demo-ing/selling the GIS

component from MapInfo for Cognos.

Note that in our current workflow, this minor fix requires re-running, re-rendering, and re-posting the umpteen PHD maps created for Lead! Moving more of the lifting from the CMS to Cognos will avoid at least some of this effort, in the future.

Portal sideshows re: Mapping – looked at demo presentation of SAS/Internet and Google Earth (GE) interaction. Nothing Earth shaking here. It's just a statistically-driven database back-end to serving data maps in a format that GE can overlay. They seemed to be able to make usable thematic map overlays (like the pre-'50 housing), which I've had trouble achieving. Bottom line: we can deliver the same kind of content, sans the "live" back-end, which is probably not necessary for the static type of data we're providing. Although Rene and I have not specifically tried this out yet. *Need to more systematically explore the options with MEGIS/Mike Smith.* FYI, the cost of using ArcGIS Server was said to be \$300/month.

Roles

Uh, I still haven't provided comments on Rene's draft document.

Water CWG – no developments.

Address Geocoding Tool – no developments, but just want to note that Mike Francoeur says it's no problem to generate the KML format used by Google Maps and Google Earth. I'm not sure if he uses stock ArcGIS Server conversion tools, or if OIT has their own library. But this bodes well for generating

Asthma/Air Surveillance – no developments.

Birth Outcomes multi-state analysis – no developments.

Arsenic in wells mapping project – no developments. *Need to follow-up with Martha Nielsen.*

Other EPHT – GIS Workshop has been deferred to next spring.

Lead

Lead Epi interviews – more phone interviews, one in-person candidate.

Online Lead materials, drafts – no time to review.

Cancer

North Deering inquiry – Had a meeting with Molly, Kathy, and Katie. Haven't heard anything lately.

Geocoding protocol – Nothing new.

CO surveillance

Analysis contract – No devs.

Program

Arsenic Exposure Study – Toenail developments.

MHDO data issues – No devs

Wiki! – OIT put up an anything-goes, 1-yr trial period Wiki. There's no support, and it's open to anyone on the Intranet. I asked them to enable file links (so we can point to files on our "S" drive, etc.). That will mostly take care of keeping materials away from others.

Eric plans to try this out as a workflow aid. I'd like to help, a little bit. *We could do the same for Portal development.*

ME-CDC

Data Workgroup – no news.

USM

Meetings, goings-on – AMS meeting with USM President, first Tuesday in December. Can't miss.

Deadlines

Demo Maps for Portal – Lead, finish soon, to start other topics!
Text, reports, etc. for Water– 12/1.

Time spent:

- 65% - EPHT Portal-related (includes Lead)
- 10% - Lead support/data request (maps for Naomi)
- 20% - Lead Epi candidate evaluation/interviews
- <5% - Water CWG: data generation, conference calls
- <5% - Geocoding Tool development
- 5% - ME-CDC, MHDO, etc. meetings
- <5% - this update

<5% - USM meetings

From: [Schwenn, Molly](#)
To: [Paulu, Chris](#); [Meyer, Katie](#)
Cc: [Decker, Kathy L.](#)
Subject: Cancer "Cluster" Meeting
Date: Monday, October 20, 2008 6:47:37 PM

Dear colleagues,

I would like to invite or request your presence at a meeting to discuss preliminary findings on cancer cases in the North Deering neighborhood of Portland. Kathy Decker has done a lot of work for MCR, picking up the work that Castine started. It will be very helpful to me and to Kathy if you can help us evaluate the findings thus far, and provide some guidance on what more should be done. School personnel have recontacted me for information.

Taking account of the schedules that I do have some knowledge of, I would like to suggest meeting on Monday, October 27 at:

3 PM to 4:30 PM.

If 10/27 is not good, how about Tuesday 10/28 at 3 PM or Thursday 10/30 AM or PM?

Please let me know in the next day or so, so that I can get a meeting room or go back to the drawing board.

Molly

P.S. Katie, once we set the date, we can decide whether this would be valuable for Anthony.

From: [Nickerson, Terri](#)
To: [Decker, Kathy L.](#)
Subject: Emailing: CasestoGeocodefromDataRequestFile052007.XLS
Date: Wednesday, June 18, 2008 11:22:03 AM
Attachments: [CasestoGeocodefromDataRequestFile052007.XLS](#)
[DeeringCases9505.XLS](#)

Kathy,

Here are two excel files (one with 1818 cases and one with 1813 cases), each with a query info tab on the worksheet. There are the only ones I have found in file folders that have the query tab.

Hope this helps. I can also send the excel file with the 4000+ cases if that would help; although it does not have the query tab with it.

Terri

From: [Schwenn, Molly](#)
To: [Meyer, Katie](#); [Paulu, Chris](#)
Cc: ["Kathy Decker"](#)
Subject: FW: Cancer "Cluster" Meeting
Date: Friday, October 24, 2008 4:20:41 PM

A late email to let you all know that we have a date that is ok with everybody and a room for the meeting about cancer in North Deering: Monday 10/27/08 (yes, next week!) at 3 to 4:30 PM in room 6b on the 1st floor of 286 Water.

Sorry for the late notice.

Molly

From: Schwenn, Molly
Sent: Monday, October 20, 2008 6:48 PM
To: Paulu, Chris; Meyer, Katie
Cc: Decker, Kathy
Subject: Cancer "Cluster" Meeting

Dear colleagues,

I would like to invite or request your presence at a meeting to discuss preliminary findings on cancer cases in the North Deering neighborhood of Portland. Kathy Decker has done a lot of work for MCR, picking up the work that Castine started. It will be very helpful to me and to Kathy if you can help us evaluate the findings thus far, and provide some guidance on what more should be done. School personnel have recontacted me for information.

Taking account of the schedules that I do have some knowledge of, I would like to suggest meeting on Monday, October 27 at:

3 PM to 4:30 PM.

If 10/27 is not good, how about Tuesday 10/28 at 3 PM or Thursday 10/30 AM or PM?

Please let me know in the next day or so, so that I can get a meeting room or go back to the drawing board.

Molly

P.S. Katie, once we set the date, we can decide whether this would be valuable for Anthony.

From: [May, Alfred](#)
To: [Paulu, Chris](#); [Smith, Andy E. \(DHHS\)](#); [Decker, Kathy L.](#); [leblanc, rene](#)
Subject: FW: MCR Meeting
Date: Tuesday, October 7, 2008 11:39:02 AM

Here is the agenda items that Molly sent to me on Friday for discussion at today's meeting (1:00 – 2:00 pm in Fish Bowl Conference Room). It is scheduled for one hour only today.

Kathy, Molly asked that I include you on the discussion about the North Deering neighborhood "cluster".
Al

Alfred J. May, Jr.

Comprehensive Health Planner/Epidemiologist
Environmental and Occupational Health Program
Maine Center for Disease Control and Prevention
286 Water Street, 3rd Floor, SHS #11
Augusta, ME 04330
207-287-3364
207-287-3981 FAX

From: Schwenn, Molly
Sent: Friday, October 03, 2008 4:46 PM
To: May, Alfred
Subject: RE: MCR Meeting

Al,

Sorry for the delay in getting these ideas to you:

North Deering neighborhood (cancer elevation in young people) with Chris and Andy present please, also Kathy?

MIHGH request – Andy or Chris, Kathy?

IT needs – if Rene is available

Molly

From: May, Alfred
Sent: Tuesday, September 30, 2008 11:08 AM
To: Schwenn, Molly
Subject: MCR Meeting

Hi Molly,

Do you have some specific issues for us to cover in next week's meeting? Since it is one hour, I would like to focus the agenda on topics that you need covered, and give a heads up to Rene or Chris if they need to have anything prepared.

Let me know.

Al

Alfred J. May, Jr.

Comprehensive Health Planner/Epidemiologist
Environmental and Occupational Health Program
Maine Center for Disease Control and Prevention
286 Water Street, 3rd Floor, SHS #11
Augusta, ME 04330
207-287-3364
207-287-3981 FAX

From: [Verrill, Castine](#)
To: [Smith, Andy E. \(DHHS\)](#); [Paulu, Chris](#)
Cc: [Schwenn, Molly](#)
Subject: FW: Pop of North Deering
Date: Friday, February 23, 2007 4:15:08 PM
Attachments: [North Deering.xls](#)

Hi Andy and Chris,

As you may have heard, we received a request to evaluate cancer and the "environmental integrity" of North Deering. North Deering is a neighborhood located within the 04103 zip code of Portland, along with many other communities. North Deering is not a town, nor does it have a specific zip code, so there is no easy way to identify the cases in our database or establish an underlying population.

The City of Portland has identified the boundaries of North Deering and matched the boundaries to two census tracts (please see email trail below). The population data of the census tracts within the zip code are attached in the Excel file. Since we don't have census tract information in our databases yet, it looks like we will have to geocode the cases in the 04103 zip code and see how many are located in the two census tracts associated with North Deering. I took a quick look at the data and most have physical street addresses.

I'd like to get your input on whether we should work with MEGIS to geocode and assign census tracts to this limited data set. We would need to proceed in a relatively short time frame. What are your thoughts?

Thanks!

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
Maine CDC
(207) 287-5190
castine.verrill@maine.gov

-----Original Message-----

From: TOHO SOMA [<mailto:TSOMA@portlandmaine.gov>]
Sent: Thursday, February 15, 2007 1:31 PM
To: Verrill, Castine
Subject: RE: Pop of North Deering

Hi again-

Attached is the updated spreadsheet, listing population for each census tract in the 04103 zip code by 5-year age group for 2000 and 1990. As you can see, the zip code area and census tracts do not align perfectly. Also, population by zip code was not captured in 1990.

Hope this helps.
Toho

Toho Soma, MPH
Public Health Information Program Coordinator
Epidemiology and Emergency Preparedness Program
Public Health Division, City of Portland
389 Congress Street, 3rd Floor

Portland, ME 04101
tsoma@portlandmaine.gov
T: 207-874-8787 / F: 207-874-8913

>>> "Verrill, Castine" <Castine.Verrill@maine.gov> 2/15/2007 11:00:55 AM >>>

Hi Toho,

Thank you very much for the population data. If possible, could I get the population for each 5-year age group (0-4, 5-9, 10-14.... 85+) for the two census tracts you identified? Also, within the zip code area of

04103 are there other census tracts? If so, would you be able to provide the population distribution for these census tracts separately?

We may only be able to look at our data by zip code (rather than census tract) and would need the corresponding population figures.

Thanks for your help. Please call if you have any questions.

Castine

-----Original Message-----

From: TOHO SOMA [<mailto:TSOMA@portlandmaine.gov>]

Sent: Thursday, February 15, 2007 10:21 AM

To: Verrill, Castine; Michael Russell

Cc: Schwenn, Molly; Bethany Sanborn

Subject: RE: Pop of North Deering

Good morning-

I've investigated various sources to try and pinpoint the population of North Deering. I first went to the North Deering Neighborhood Association's website, where they had a map of what they considered to be North Deering's boundaries. I then concluded that they were based on the US Census Bureau's census tracts (#21.01 and 22 in Cumberland County).

Here's the glitch--census tract populations are only counted every 10 years, so I could only get population data for 1990 and 2000. Fortunately, they also enumerate by age. The attached spreadsheet shows the population for each age group requested for the two census years. Annual population estimates are only done down to the city level, and even then, not separated by age group.

If you have any questions or if I can be of any further assistance, please let me know.

Thanks,

Toho

Toho Soma, MPH
Public Health Information Program Coordinator
Epidemiology and Emergency Preparedness Program
Public Health Division, City of Portland
389 Congress Street, 3rd Floor
Portland, ME 04101
tsoma@portlandmaine.gov
T: 207-874-8787 / F: 207-874-8913

From: [Verrill, Castine](#)
To: [Paulu, Chris](#)
Cc: [Schwenn, Molly](#); [Smith, Andy E. \(DHHS\)](#)
Subject: N. Deering
Date: Tuesday, May 8, 2007 2:54:20 PM

There are approximately 1,800 cases to geocode for the 04103 zip code from 1995-2005.
Thanks for your input on the timeline for completing the process.

Castine Verrill, MS

Epidemiologist

Maine Cancer Registry

Maine CDC

(207) 287-5190

castine.verrill@maine.gov

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From: [Verrill, Castine](#)
To: [Paulu, Chris](#)
Subject: N. Deering
Date: Friday, April 4, 2008 12:47:39 PM

Hi Chris,

It is crazy to think I only have a week and a half of work left! I know you are busy, but the North Deering concern is something I hate to leave unfinished. I was wondering if you have made progress on the mapping and census tract application for the Portland data. Also, do you have access to the Census 2000 gender/age-specific population files for the Census Tracts within Portland?

Thanks for your help!
Castine

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
MECDC, DHHS
(207) 287-5190
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From: [Kathy Decker](#)
To: [Meyer, Katie](#); [Paulu, Chris](#); [Schwenn, Molly](#); [Molly Schwenn](#)
Subject: North Deering Analysis
Date: Sunday, March 15, 2009 4:20:16 PM
Attachments: [NDeering_DataReport032009.doc](#)
[NDeering_rpttbl.xls](#)

Dear all,

I am attaching a draft report of the analysis to date for North Deering investigation. The intent of this report is just to consolidate the findings in terms of comparison across geography. The intent is not to talk about or discuss 'why' we may be finding differences or identify any environmental risk factors or exposures. You will find two documents one a .doc that is the report body (very short only 4 pages) and an .xls that includes all the tables referenced in the report.

At our last meeting I was asked to look at the distribution of 0-19 year old cancers, represented here:

Number of malignant cancer cases, age 0-19 years, by year of diagnosis, 1995-2004

Year	North Deering	Portland
19950		1
19960		4
19970		3
19982		7
19992		3
20003		4
20011		2
20021		2
20030		2
20041		1
10		29

I was also asked to provide the comparison of Portland excluding North Deering. The report presents information on the comparison of Portland excluding North Deering. I was also asked to calculate the rates of melanoma, thyroid, and kidney cancers in the 20-44 year old age group. This is also included in the report. Lastly, I was asked to pull the myeloid and monocytic leukemia counts for 2005 & 2006; Maine 4 in 2005 and 5 in 2006, Cumberland County 0 in 2005 and 2 in 2006 (these were pulled from SeerStat, so not sure if the 2006 data is up to date).

I know it's been quite some time but I believe these were all my tasks. Finally, Molly has asked me to set up a time for us to meet again and discuss if we need anything further to make some kind of statement. She needs to meet with the North Deering folks and wants your input on how to present the information collected.

Anyway, I would like to set up a meeting with all of us for Tuesday, March 31st at 3:30 or 4 pm if you are available. Please respond and let me know your availability. Thank you and let me know if you have any questions.

Regards,
Kathy
on

NORTH DEERING NEIGHBORHOOD ANALYSIS

Introduction

In MONTH YEAR, a group of concerned citizens contacted the Maine Cancer Registry with concerns regarding the number of persons diagnosed with cancer in Portland's North Deering neighborhood. The following information is based on analysis completed to assist with the investigation into the number of incident cancer cases in the North Deering neighborhood. Of particular interest was the suspected number of incident cases found in the youth/young adult population. The intent of the analysis was to see whether the number of incident cancer cases in the North Deering neighborhood differed from what you would expect to find in a Maine population.

Methods

Demographic information on the cases identified by the concerned citizens was procured and cross checked with the Maine Cancer Registry tumor database (IMPAC) for accuracy. There were 3,398 incident tumor cases identified by the Maine Cancer Registry for Portland during the time period of 1995-2004 using IMPAC. Cumberland County and Maine State cases counts were retrieved using SEER Stat software.

Portland case addresses were geocoded according to their latitudinal and longitudinal coordinates using XXX software. Of the 3,398 tumor cases, 136 (4.0%) were not geocoded due to either a missing address or post office box listing and excluded from the analysis. Using GIS software, cases were then categorized and coded into one of the Portland Census tracts.¹

Census 2000 data were used to estimate the population of North Deering and Portland.¹ Post-2000 and inter-censal estimates were used for Cumberland County and Maine State estimates (1995-1999 and 2000-2004, respectively).

Cases were stratified by geography, age at diagnosis, and selected major cancer site. Four strata were constructed for the analysis; North Deering, Portland (excluding North Deering hereafter referred to as just "Portland"), Cumberland County, and Maine State. Two census tracts (23005002101 and 23005002200) were identified as belonging to the North Deering neighborhood. The remaining seventeen census tracts were identified as belonging to the strata named Portland. Age groups used in the analysis were 0-19 years, 20-44 years, 45-64 years, 65-74 years, 75-84 years, and 85 plus years. Cancers were categorized according to the SEER incident cancer site recode definitions.²

In the youngest age group, 0-19 years, cancers were grouped and analyzed by the following categories; all cancers, lymphocytic leukemia, acute lymphocytic leukemia, myeloid and monocytic leukemia, other leukemia, Hodgkin lymphoma, Non-Hodgkin

¹ Portland Census tracts: 23005000100, 23005000200, 23005000300, 23005000500, 23005000600, 23005001000, 23005001100, 23005001200, 23005001300, 23005001500, 23005001700, 23005001800, 23005001900, 23005002001, 23005002002, 23005002101, 23005002102, 23005002200, 23005002300.

lymphoma, and brain cancer. All of the other age groups had incident cancer cases analyzed according to the following groups; all cancers, lung, colon, breast (female only), and prostate cancer. Three additional cancer sites were analyzed for the 20-44 year old age group; melanoma, thyroid, and kidney.

Rates and accompanying 95% confidence intervals were calculated using SAS software for the North Deering and Portland analysis³. The expected number of cases was calculated for North Deering based on each of the three remaining strata, i.e. Portland, Cumberland County and Maine State.

Results

Between 1995 and 2004 there were 10 cases of incident malignant cancers (insitu included for brain cancers) found in persons 0-19 years of age in the North Deering neighborhood, 19 cases in Portland, 139 cases in Cumberland County, and 609 cases in total for Maine (Table 1). Of the ten cases found in North Deering, eight are accounted for in the individual incident site categories presented next. One case each of respiratory and endocrine site cancers were not included in the individual site analysis (data not shown).

In persons aged 0-19 years there were no differences in the observed number of cases when using Cumberland County or Maine as the basis for acute lymphocytic, and brain cancer (Tables 2-3). North Deering experienced higher than the expected number of cases of myeloid and monocytic leukemia (17.0 per 100,000), and Hodgkin lymphoma (17.0 per 100,000) compared to Cumberland County (1.2 and 2.3 per 100,000, respectively) and Maine (1.0 and 1.6 per 100,000, respectively)(Tables 4-5). There were no cases of other leukemia or Non-Hodgkin lymphoma reported in North Deering during the same time period (1995-2004)(Table 6).

For the same time period, there were 53 cases of incident malignant cancers found in persons 20-44 years of age in North Deering neighborhood, 198 cases in Portland, 1,061 cases in Cumberland County, and 5,081 in total for Maine (Table 7). Of the 53 cases found in North Deering, 31 are accounted for in the individual incident site categories presented. Of the twenty-two incident cancers not compared in the following analysis, 6 were lymphoma, 3 cervix or ovary, 2 oral cavity, 3 digestive, 1 respiratory, 1 brain, 2 testis, 1 invalid (data not shown)(*I know I'm missing 3 cancers in the group*).

In persons aged 20-44 years there were no differences in the observed number of cases when using Portland, Cumberland County or Maine as the basis for lung, colon, breast, and prostate cancers (Tables 8-11). North Deering experienced higher than the expected number of cases of melanoma (26.2 per 100,000) compared to Portland, Cumberland County, and Maine (7.2, 10.8, and 12.4 per 100,000, respectively)(Table 12). North Deering experienced a higher number of thyroid and kidney cancers (17.5 and 8.7 per 100,000, respectively) compared to Portland (6.0 and 1.8 per 100,000, respectively)(Tables 13-14). There were no observed differences in the expected number of thyroid or kidney cancer in North Deering compared to Cumberland County or Maine.

For the same time period in persons aged 45-64 years, there were 162 cases of incident malignant cancers in North Deering neighborhood, 705 cases in Portland, 4,386 cases in Cumberland County, and 23,016 in total for Maine (Table 15). Of the 162 incident cases in North Deering, 94 are accounted for in the site specific analysis.

There were no differences in the observed number of incident cases of lung, colon, and prostate cancer in North Deering compared to Portland, Cumberland County, or Maine (Tables 16-17, 19). The number of breast cancer cases was higher than expected in North Deering (340.4 per 100,000) compared to Portland (251.1 per 100,000), Cumberland County (279.9 per 100,000), and Maine (262.0 per 100,000)(Table 18).

In persons aged 65-84 years there were 328 cases of incident malignant cancers in the North Deering neighborhood, 1,212 cases in Portland, 6,876 cases in Cumberland County, and 38,593 cases in Maine (Table 20). Of the 328 incident cases in North Deering, 180 are accounted for in the site specific analysis.

No differences were observed in the number of incident cases of colon cancer in North Deering compared to Portland, Cumberland County, and Maine (Table 22). North Deering had lower than expected numbers of lung cancer (384.1 per 100,000) compared to Portland (469.6 per 100,000), Cumberland County (427.7 per 100,000), and Maine (443.4 per 100,000)(Table 21). The number of incident cases of prostate cancer were also lower in North Deering (705.7 per 100,000) compared to Cumberland County (806.4 per 100,000) and Maine (980.6 per 100,000)(Table 24). The number of newly identified breast cancers in North Deering (456.8 per 100,000) were higher when compared to Portland (371.0 per 100,000)(Table 23).

In persons aged 85 years or more, there were 89 cases of incident malignant cancers in the North Deering neighborhood, 248 cases in Portland, 1,171 cases in Cumberland County, and 5,635 cases in Maine from 1995 to 2004 (Table 25). Of the 89 incident cases identified, 42 are accounted for in the site specific analysis.

There were no differences in the observed number of cases of breast cancer in North Deering compared to Portland, Cumberland County, or Maine (Table 28). There were fewer cases of colon cancer in North Deering (288.2 per 100,000) compared to Portland (456.2 per 100,000)(Table 27). In North Deering there were higher than expected numbers of lung cancer (432.2 per 100,000) compared to Portland (256.7 per 100,000), Cumberland County (320.2 per 100,000), and Maine (306.9 per 100,000)(Table 26). Prostate cancer was higher in North Deering (1,020.4 per 100,000) compared to Portland (393.0 per 100,000)(Table 29).

Limitations

There are a few limitations for this analysis. First, the small number of cases identified in the North Deering neighborhood limits the precision of our estimates. Additionally, we

identified that of the 136 cases not geocoded and excluded from the analysis 132 were malignant tumors, therefore we do not know if they were missing at random across both site and geography (North Deering versus Portland)(data not shown). In the analysis we identified a few cancer sites that only differed between the North Deering neighborhood and Portland such as, prostate cancer in 65-84 and 85 plus years of age, breast cancer 65-84 years of age, thyroid and kidney cancer in 20-44 year olds. Each of these may have been affected by the missing cases. For example, in the missing cases we identified 10 prostate cancers in men 45-64 years of age, if only 1 of those cases is located in North Deering then the difference in observed numbers between North Deering and Portland goes away (data not shown). This demonstrates the instability of working with small numbers and the effect even small shifts in unknowns may have on the ultimate analysis.

Finally, the intent of this report is to present findings of base analysis only. The report is not a complete description of the cancer types and their relationship to or identification of potential environmental exposures.

¹ Census Bureau. American Factfinder Census 2000 dataset. Available URL: <http://www.census.gov>.

² National Cancer Institute (NCI), Surveillance Epidemiology and End Results (SEER) Incidence site recode. Available at: <http://seer.cancer.gov/siterecode/index.html>

³ SAS Institute, Inc. (2002-2003). *SAS Release 9.1.3*. Cary, NC.

From: [Smith, Andy E. \(DHHS\)](#)
To: [Paulu, Chris](#)
Subject: RE: CP - work update, weeks ending 07/11/2008
Date: Monday, July 14, 2008 1:31:40 PM

Thanks Chris, these are really helpful. See some comments below. Add a section for deadlines.

Andrew E. Smith, SM, ScD
State Toxicologist
Director, Environmental and Occupational Health Programs
Maine Center for Disease Control and Prevention
Department of Health and Human Services
11 State House Station
Augusta, ME 04333
Tel: (207) 287-5189
Fax: (207) 287-3981

From: Paulu, Chris
Sent: Monday, July 14, 2008 1:03 PM
To: Smith, Andy E. (DHHS)
Subject: CP - work update, weeks ending 07/11/2008

On my Plate

EPHT – Portal development

- Lead maps, reporting

- Water CWG – data, conference calls, sharing agreement
- Address Geocoding Tool
- Asthma/air surveillance
- Cognos assistance
- Arsenic in wells mapping project
- Birth outcomes multi-state project

Lead - data requests (Auburn-Lewiston HUD proposal)

- address cleaning/geocoding, 2003-2007

- Battelle risk prediction project

Cancer – North Deering inquiry

- geocoding protocol

Program – Arsenic exposure study

ME-CDC – Data Access Workgroup

USM – misc.

This week's activities

EPHT

Portal development – lots of discussions. Some assistance/experimentation with Cognos. Bottom line for Cognos maps: Lisa can join data successfully; number of categories, colors and cutpoints are customizable, but with none of the automatic aids of ArcGIS, nor the symbology options (scaled dots, dot density). I don't believe there is any layering or zooming functionality, and there certainly isn't any capability to display an ad hoc set of latitude/longitude points. Right now we have county map display only. I need to generate an import file for PH Districts, towns, etc. We should make a list of display layers; it would be best to do this before my 30 day MapInfo demo expires.

Lead maps, reports – meeting with Kathy, Naomi. I have a summary of maps to be presented. I will forward it to the four of us for review. (See Lead topic below for discussion of address cleanup and geocoding).

Water CWG – data sharing agreement with DWP will be signed shortly. Revised NCDM code was implemented and XML data files generated. It seemed to work smoothly, and XML validated. Prior to submission I will attend a conference call about sensible QC steps, and will explore the data for errors (original and calculation glitches). Final submission slated for Thursday, 7/17. *Metadata* – I'm not planning to update metadata yet, based on a chat with Rish V. last week. There was some conflicting information from SND though (via Lisa).

Address Geocoding Tool – I have tested a single-address web interface, and provided comments on a 0.0.1 version. I have a meeting today (7/14) with Mike F. to discuss (I preferred email, but he insisted on face-to-face). I think the Tool sucks considerably less than it used to. They have added an option to use NAVTEQ data, and it reports back to the user the “nearest match” when a suitable match is not found. However, much of the verbiage is still bogus, and Lisa and Cindy found it to be pretty lame. It concerns me that my opinion is less harsh: it could be that my expectations of OIT work have hit the basement level. [Keep pushing them Chris.](#)

Asthma/Air Surveillance – no developments

Birth Outcomes multi-state analysis – no news

Arsenic in wells mapping project – Charlie Culbertson will take this over. Need to summarize what I've done, outline what he can/should do, prior to meeting.

Lead

Data request – Auburn-Lewiston data request for HUD grant. Tasks: devising census-unit based target area based on City request and inspection of EBL locations (2005-2006); geocoding of *screenees*, since HUD requests # of children tested and <10 children in target area and Auburn-Lewiston as a whole. There were ~2,000 screening-related addresses. Dual-source geocoding reduced the manual inspection load to about 5% (100 addresses).

Address cleaning/geocoding, general data QC – lots of discussion in Portal meeting; reviewed species of address validation “errors” (by direct inspection, and Lisa’s summary). I have some general observations on this – not yet circulated – and will take the lead to make sure it gets done (with assistance from Cindy, May, whoever).

Battelle risk prediction project – attended webinar. Generally impressed that the people working on this project are intelligent, capable, honest (“we’ll take whatever data we can get our grubby hands on” – I appreciated that line). They gave us a pre-print article (haven’t read it thoroughly). They’re project is all about “risk prediction” and I have reservations about this endeavor - as in, “what’s the point?” They talked about using the output “predicted risk” as a tool of intervention. But if the predicted risk far exceeds the observed EBLs, what does that tell us? Or vice-versa? Perhaps this works better in Massachusetts where they have extensive building inspection data. Not clear to me whether we should (and legally can) do this. Difficult to see what we gain, and while our statutory language will allow us to share address data, I am not sure we can share other data with them that would make it any easier to identify an individual. So, I suppose I need to better understand what they specifically want from us, data wise.

Cancer

North Deering inquiry – No developments really, but I know that I’ll be needed to check and review what Kathy does, perhaps generate some maps, etc., and this is a pretty big little project (I think). If it is a pretty big project, ..who is going to do this?

Geocoding protocol – I consider my work with Lead addresses and with the Address Tool as all feeding into a geocoding protocol that Cancer will use.

Program

Arsenic Exposure Study – No developments. It would be nice to schedule a future time to work on this. – Some developments that I may not have told you about. I have (over the past few months) collected a number of toe nail samples from my self. I have sent them to the lab to be weighed to see if enough mass to either split in half or three-ways. Looks like answer is yes, but for big toe nail samples only. HETL is willing to assist us with a pilot soaking study. Plan is to take a big toe nail sample (3 or 4 of them), split into thirds: one part is saved as control, one is soaked for 48-hours in 500 ppb arsenate, the third part is soaked in 500 ppb arsenite for 48 hours. Samples are then shipped off to Dartmouth for analysis. This, I am hoping will tell us whether we have a toe nail issue or not. Any hint we do, then we move forward with paper with urine data only, as we try to find a way to undertake a more robust study. If no evidence of any adsorption from soaking,we write paper with both results and scratch our head. Sound OK? Hope to have pilot study run in August.

ME-CDC

Data Workgroup – You were there. Meeting + trial experimentation with the demo Data Access site. I’m still not sure what to make of it in general, but it strikes me as a

different approach to some similar information access objectives as the EPHT Portal. So it has complementary advantages and disadvantages. Our groups can learn from each other's efforts.

USM

Meetings, goings-on – No developments (it's summer), aside from travel budgeting and Overhead bargaining.

[ADD Pending Deadlines section](#)

Time spent:

- 40% - EPHT Portal-related
- 35% - Lead support/data request
- 15% - Water CWG: data generation, conference calls
- 5% - Geocoding Tool development
- <5% - ME-CDC Data Access Workgroup
- <5% - this update

From: [Smith, Andy E.](#)
To: [Schwenn, Molly](#); [Verrill, Castine](#); [Mills, Dora A.](#)
Cc: [Poirier, Barbara](#)
Subject: RE: Cancer Study in North Deering neighborhood in Portland
Date: Tuesday, February 13, 2007 9:44:39 PM

I guess I am behind the times, ... is this the news?

A Portland neighborhood is going under the microscope. The Maine center for disease control is conducting a cancer study of the North Deering section of Portland. The request was made by people at Iyseth Elementary school after a high number of students and parents were diagnosed with cancer over the last decade. "I think whenever you have children who have been diagnosed with cancer which we have in this bldg and actually passed away it becomes a concern we don't know if it's an unusual number for an environment this size." The cancer study is expected to take a few weeks. Over the past 20 years, only one cancer cluster has been identified in Maine. That was an outbreak of brain cancer in Fairfield a few years ago.
- what is going on, ...is ME CDC conducting a study (or cancer cluster investigation)?

- Andy

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-----Original Message-----

From: Schwenn, Molly
Sent: Tuesday, February 13, 2007 4:25 PM
To: Verrill, Castine; Mills, Dora A.; Smith, Andy E.
Cc: Poirier, Barbara
Subject: FW: Cancer Study in North Deering neighborhood in Portland

This is really getting out of control (thanks, I think, to the television report last night). I can respond to this woman if no one else wants to. (With mostly NOs.) Volunteers?
Molly

-----Original Message-----

From: [REDACTED]
Sent: Tuesday, February 13, 2007 4:17 PM

To: Schwenn, Molly

Subject: Cancer Study in North Deering neighborhood in Portland

Hi,

I'm planning to buy a home in the [REDACTED] and I saw a news report on WGME 13 last evening regarding a cancer study that's being done. Where can I get more information about this? Should I be testing the water and soil in this home before I close on it? Is there anything else I can be testing? Do you know of a professional that does this type of testing?

Thanks,

[REDACTED]

From: [Paulu, Chris](#)
To: [Schwenn, Molly](#)
Subject: RE: Cancer "Cluster" Meeting
Date: Tuesday, October 21, 2008 7:51:04 AM

Hi,
I am available for any of those times.
CP

From: Schwenn, Molly
Sent: Monday, October 20, 2008 6:48 PM
To: Paulu, Chris; Meyer, Katie
Cc: Decker, Kathy
Subject: Cancer "Cluster" Meeting

Dear colleagues,

I would like to invite or request your presence at a meeting to discuss preliminary findings on cancer cases in the North Deering neighborhood of Portland. Kathy Decker has done a lot of work for MCR, picking up the work that Castine started. It will be very helpful to me and to Kathy if you can help us evaluate the findings thus far, and provide some guidance on what more should be done. School personnel have recontacted me for information.

Taking account of the schedules that I do have some knowledge of, I would like to suggest meeting on Monday, October 27 at:
3 PM to 4:30 PM.

If 10/27 is not good, how about Tuesday 10/28 at 3 PM or Thursday 10/30 AM or PM?

Please let me know in the next day or so, so that I can get a meeting room or go back to the drawing board.

Molly

P.S. Katie, once we set the date, we can decide whether this would be valuable for Anthony.

From: [Meyer, Katie](#)
To: [Schwenn, Molly](#); [Paulu, Chris](#)
Cc: [Decker, Kathy L.](#)
Subject: RE: Cancer "Cluster" Meeting
Date: Tuesday, October 21, 2008 9:53:10 AM

Dear Molly,

Monday works for me—Tuesday and Thursday do not, though I could call in on Thursday.

Katie

From: Schwenn, Molly
Sent: Monday, October 20, 2008 6:48 PM
To: Paulu, Chris; Meyer, Katie
Cc: Decker, Kathy
Subject: Cancer "Cluster" Meeting

Dear colleagues,

I would like to invite or request your presence at a meeting to discuss preliminary findings on cancer cases in the North Deering neighborhood of Portland. Kathy Decker has done a lot of work for MCR, picking up the work that Castine started. It will be very helpful to me and to Kathy if you can help us evaluate the findings thus far, and provide some guidance on what more should be done. School personnel have recontacted me for information.

Taking account of the schedules that I do have some knowledge of, I would like to suggest meeting on Monday, October 27 at:
3 PM to 4:30 PM.

If 10/27 is not good, how about Tuesday 10/28 at 3 PM or Thursday 10/30 AM or PM?

Please let me know in the next day or so, so that I can get a meeting room or go back to the drawing board.

Molly

P.S. Katie, once we set the date, we can decide whether this would be valuable for Anthony.

From: [Wigand, Debra A.](#)
To: [Schwenn, Molly](#)
Cc: [Zaremba, Maryann M.](#)
Subject: RE: Children's cancer rates
Date: Thursday, August 30, 2012 8:10:15 AM

I agree that she seems to be approving this message to the constituent. Yes, I would work directly with the individual and if there are questions that get to areas you want advice about how to answer, seek out that help.

Debra A. Wigand, MEd, MCHES
Director, Division of Population Health
Maine CDC/DHHS
Key Plaza, 4th Floor
11 SHS
Augusta, ME 04333-0011
Phone: 207-287-4624
Fax: 207-287-4631
Email: debra.a.wigand@maine.gov

Web: www.mainepublichealth.gov
Facebook: www.facebook.com/MaineCDC
Twitter: [@MEPublicHealth](https://twitter.com/MEPublicHealth)
Blog: <http://mainepublichealth.blogspot.com>

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-----Original Message-----

From: Schwenn, Molly
Sent: Wednesday, August 29, 2012 5:49 PM
To: Wigand, Debra A.
Cc: Zaremba, Maryann M.
Subject: FW: Children's cancer rates

Deb,

I'm assuming, given Chris' response, that I do not need to get permission to speak with this constituent nor to review my response. Let me know if this assumption is not correct.

Molly

-----Original Message-----

From: Zukas, Christine
Sent: Wednesday, August 29, 2012 5:45 PM
To: Schwenn, Molly
Subject: Re: Children's cancer rates

Very nice response, Molly. Thank you!

----- Original Message -----

From: Schwenn, Molly
Sent: Wednesday, August 29, 2012 05:42 PM
To: [REDACTED]
Cc: Zukas, Christine

Subject: RE: Children's cancer rates

Dear [REDACTED]

Chris Zukas forwarded your inquiry to me. I will be happy to speak with you about the findings of the North Deering neighborhood cancer evaluation. I have just located the statistical work done 2008 - 2009 and would like to review it before we talk. No additional work has been done since 2009. I will also review data produced by the first epidemiologist who worked with the North Deering data. She no longer works at the Cancer Registry nor in Maine, since 2008, so she is not available.

If you could give me your phone number and some times on Friday (8/31) or next week that you are available, I will call you. Let me know if you prefer that I give you a time to call me at the Cancer Registry.

Sincerely,

Molly Schwenn, M.D.
Director, Maine Cancer Registry
Division of Population Health

----- Original Message -----

From: [REDACTED]
Sent: Tuesday, August 28, 2012 11:18 AM
To: Zukas, Christine
Subject: Children's cancer rates

Hi-

A few years ago I spoke to the woman who was our state health expert regarding a news report on higher childhood cancer rates in the north deering neighborhood in Portland. I live in [REDACTED] and was wondering if you have any further studies that have been done on this issue. I now have [REDACTED] and this really concerns me. Is there anyone that I can speak with about this?

[REDACTED]

Sent from my iPhone

From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Subject: RE: Genetics Inst
Date: Friday, April 11, 2008 10:15:44 AM

Hi,

Bad news/good news/etc.

 We cancelled our vacation for next week.

The good news is that, if all goes well, next week will be a pretty regular work week for me. So I'll have more time to work on the lung ca and North Deering projects.

I didn't really want to go on vacation anyway!

CP

From: Verrill, Castine
Sent: Friday, April 11, 2008 8:54 AM
To: Paulu, Chris
Subject: Genetics Inst

Good morning Chris,

I hope you had a good trip... and I'm sure you are looking forward to vacation next week. Thank you so much for getting the data geocoded for the N. Deering investigation. I plan to look at the data today and will be in touch if I have questions.

Next Tuesday we have a conference call with the Genetics Institute and would like to present the lung cancer data. Attached is the latest town distribution, which now includes Somerset and Waldo County, in addition to Aroostook, Hancock, Penobscot, Piscataquis, and Washington. Will you have time to map this data? I tried to get an official count of the number of towns (minor civil divisions) in the 7 counties of interest so we could calculate the percentage of towns that have 5 or more cases. The file I have includes all the Tracts (e.g. T10 R7 WELS). I suppose we should count these as towns, correct? The count was 669. Does that seem right to you?

I'm sorry if this late notice puts a wrinkle in your day. Please let me know if this will have to wait.

Thanks!
Castine

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
MECDC, DHHS
(207) 287-5190
castine.verrill@maine.gov

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From: [Mills, Dora A.](#)
To: [Lovejoy, Elaine](#); [Schwenn, Molly](#); [Apedoe, Netta](#); [Smith, Andy E. \(DHHS\)](#)
Subject: RE: Mail Question
Date: Monday, December 18, 2006 10:57:44 AM

Elaine - can you please make and send copies to all of the above staff?

Molly/Netta - this appears to be a letter primarily asking for a cancer cluster reponse, but I'm asking Elaine to send it to Andy too, in case he feels they have a role. If the cancer cluster determination and response is going to take more than 3 weeks (which it usually does) can you pls write and send them a brief response saying you will be getting in touch with them and looking into this? Pls cc me on all your correspondence with them.

Thank you!! Dora

From: Lovejoy, Elaine
Sent: Mon 12/18/2006 9:50 AM
To: Mills, Dora A.
Subject: Mail Question

Good morning!!

You got a request in this morning's mail, from the Harrison Lyseth Elementary School, for a review of the "environmental integrity of the No. Deering neighborhood in Portland" - because of a high incidence of cancer. Should I make copies for anyone (like Molly Schwenn, Netta Apedoe, Andy Smith, ??) or do anything with this letter?

Talk soon :)

Elaine

From: [Verrill, Castine](#)
To: [Paulu, Chris](#)
Cc: [Schwenn, Molly](#)
Subject: RE: N. Deering
Date: Tuesday, April 15, 2008 6:30:52 PM

Hi Chris,

I looked at the file referenced below. Thank you for re-linking the databases to include the original data items from the confidential file. You did include 2000 Census population figures by census tract; however, the age groupings are not the same as what we have for our cases. Is it possible to get the population data by 5-year age groups (e.g. 0-4, 5-9, 10-14...85+)?

Castine

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
MECDC, DHHS
(207) 287-5190
castine.verrill@maine.gov

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From: Paulu, Chris
Sent: Friday, April 04, 2008 5:12 PM
To: Verrill, Castine
Cc: Nickerson, Terri; Schwenn, Molly
Subject: RE: N. Deering

Castine,

Alright, I've got something for you to look at. It's in our cross-share, which you *should* have access to (if not, Terri will):

[\\dhshsa6fsom001\mcr-epht](#)

- and then go into the "North Deering Project" folder.

I created an Excel file, "Portland_final_list_with_tract_data.xls" which has a table of the cancer records, with Tract IDs, and a separate table of Tract demographics (this comes directly from MEGIS).

The final selection are all those addresses within a Portland census tract OR having an unmapped address that was unknown or a PO Box (but appeared to be in Portland). I left those in so you could include the "non-mapped, but in Portland" in the final tallies.

I have to admit that time has not allowed me to really pore over the final locations/data. In the final

geocoding process I did do a cross-check with an automated geocoding run, and I resolved a few more of the non-geocoded ones. I think it's pretty clean, and I trust and know that Terri did a good job fixing up the recalcitrant addresses.

It looks to me like there are 3,267 out of 3,398 mapped (96%), for the Portland-assigned addresses. That's pretty good. The remaining 4% are mostly PO Boxes or "Unknowns" with very few having an inscrutable address.

The final source data are in an Access file. I'm guessing you won't want to mess with that.

A project map is in an ArcGIS file, "Portland_project_map.mxd". You might want to take a look at that.

I hope this is enough for you to take it away! Any issues, of course...contact me.

I won't be in the office Monday, I'm away at a conference Tuesday-Thursday. Back on Friday. But then gone on vacation the following week!

So just email...

CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 1:03 PM
To: Paulu, Chris
Subject: RE: N. Deering

That's correct. Thank you so much for working on this with everything else that's going on.

I still need to get you the additional county data for the lung data. Sorry for the delay.

From: Paulu, Chris
Sent: Friday, April 04, 2008 1:00 PM
To: Verrill, Castine
Subject: RE: N. Deering

Let me just confirm one thing though: this is Portland only analysis, right? The only reason we have Falmouth, etc., is due to overlap, mistaken zips, and so on. Right?

Okay.

CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 12:48 PM
To: Paulu, Chris
Subject: N. Deering

Hi Chris,

It is crazy to think I only have a week and a half of work left! I know you are busy, but the North Deering concern is something I hate to leave unfinished. I was wondering if you have made progress on the mapping and census tract application for the Portland data. Also, do you have access to the Census 2000 gender/age-specific population files for the Census Tracts within Portland?

Thanks for your help!
Castine

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
MECDC, DHHS
(207) 287-5190
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From: [Verrill, Castine](#)
To: [Paulu, Chris](#)
Cc: [Schwenn, Molly](#); [Nickerson, Terri](#)
Subject: RE: N. Deering
Date: Friday, May 11, 2007 6:06:04 PM

Hi Chris,

Thank you for looking at the census tracts and identifying additional streets to be included in the analysis. I have pulled a file of all cases in the 04103 zip code as well as the specified streets in the 04102 zip code. We are currently reviewing the file for any duplicate cases. Terri will be taking a look at the file for address standardizing issues and may want to speak with you about how to move forward with the process. I have yet to log into Citrix to make sure ArcGIS runs o.k. - Terri has it on her computer if one of you wants to check into it.

I will be out of town until next Friday, the 18th. Could we meet on Tuesday the 22nd for some ArcGIS training? I'm free all day. I'm also available all day on the 24th if that works better.

Thanks and have a great weekend! Whee :-)

Cas

-----Original Message-----

From: Paulu, Chris
Sent: Wednesday, May 09, 2007 12:17 PM
To: Verrill, Castine
Subject: RE: N. Deering

Hi Cas,

I'm going to pretend we're going ahead with this North Deering project. I was looking at the Census Tracts yesterday, and there appears to be a triangular sliver of zipcode 04102 that extends into one of the tracts of interest. To be safe, I think we should add a selection of those streets to the selection of the main zipcode. Here they are...

Add the following streets from Zipcode 04102:

- Holm Ave
- Birchwood Dr
- Aspen Dr
- Pinecrest

I would do this either by sorting the street names to look for these names and their variants, or just filtering using the name alone (i.e., "holm", "birchwood", etc.). Whatever will have the best chance of catching those streets, basically.

Other than that, the only prep work is to create an Access file that contains all the addresses. Also, log in to Citrix and make sure the ArcGIS stuff runs properly.

Then we can arrange a time to get started. Whee!

CP

From: Verrill, Castine
Sent: Tuesday, May 08, 2007 2:54 PM
To: Paulu, Chris

Cc: Schwenn, Molly; Smith, Andy E.
Subject: N. Deering

There are approximately 1,800 cases to geocode for the 04103 zip code from 1995-2005.

Thanks for your input on the timeline for completing the process.

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
Maine CDC
(207) 287-5190
castine.verrill@maine.gov

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Subject: RE: N. Deering
Date: Wednesday, May 9, 2007 12:17:21 PM

Hi Cas,

I'm going to pretend we're going ahead with this North Deering project. I was looking at the Census Tracts yesterday, and there appears to be a triangular sliver of zipcode 04102 that extends into one of the tracts of interest. To be safe, I think we should add a selection of those streets to the selection of the main zipcode. Here they are...

Add the following streets from Zipcode 04102:

- Holm Ave
- Birchwood Dr
- Aspen Dr
- Pinecrest

I would do this either by sorting the street names to look for these names and their variants, or just filtering using the name alone (i.e., "holm", "birchwood", etc.). Whatever will have the best chance of catching those streets, basically.

Other than that, the only prep work is to create an Access file that contains all the addresses. Also, log in to Citrix and make sure the ArcGIS stuff runs properly.

Then we can arrange a time to get started. Wheel!

CP

From: Verrill, Castine
Sent: Tuesday, May 08, 2007 2:54 PM
To: Paulu, Chris
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Subject: N. Deering

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From: [Paulu, Chris](#)
To: [Smith, Andy E. \(DHHS\)](#); [Verrill, Castine](#)
Cc: [Schwenn, Molly](#); [Nickerson, Terri](#)
Subject: RE: N. Deering
Date: Wednesday, May 9, 2007 12:10:44 PM

Yes, 1,800 addresses is over my threshold for "just do it".

I'm going to guesstimate that 70% can be geocoded easily (taking just minutes), and 30% will pose a problem. That leaves 540 addresses to be inspected, cleansed, and perhaps geocoded interactively. I'll allow 5 minutes apiece, although once in the swing of things a person would probably do 20-30/hour. On the high side, this is ~35 hours of manual labor.

My former self has convinced my present self that this is still a golden opportunity for an in-house effort that will positively cement new and useful skills. Are Cas and Terri up for this?

For my part, I would expect to spend 2 hours on set up and training at Castine's computer, plus another 2 hours for later questions/consultation spread out over a week or two.

How does that sound?

CP

From: Smith, Andy E.
Sent: Tue 5/8/2007 9:24 PM
To: Verrill, Castine; Paulu, Chris
Cc: Schwenn, Molly
Subject: RE: N. Deering

Hmmm, sounds like a lot to me. Chris, ...what is your estimate of time needed to geocode these cases?

- Andy

Andrew E. Smith, SM, ScD
State Toxicologist and Director,
Environmental and Occupational Health Program
Maine Center for Disease Control and Prevention
11 State House Station
Augusta, ME 04333
Tel: (207) 287-5189
Fax: (207) 287-3981
Pager: (207) 759-7156
email: andy.e.smith@maine.gov

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Sent: Tuesday, May 08, 2007 2:54 PM
To: Paulu, Chris
Cc: Schwenn, Molly; Smith, Andy E.

Subject: N. Deering

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From: [Smith, Andy E. \(DHHS\)](#)
To: [Verrill, Castine](#); [Paulu, Chris](#)
Cc: [Schwenn, Molly](#)
Subject: RE: N. Deering
Date: Tuesday, May 8, 2007 9:24:26 PM

Hmmm, sounds like a lot to me. Chris, ...what is your estimate of time needed to geocode these cases?

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From: [Schwenn, Molly](#)
To: [Paulu, Chris](#)
Subject: RE: N. Deering
Date: Friday, April 4, 2008 5:27:24 PM

Thank you, Chris. I can't get in until Terri or Castine are here on Monday, but I know it will be exciting. (Light at the end of the tunnel, perhaps?)
Molly

From: Paulu, Chris
Sent: Friday, April 04, 2008 5:12 PM
To: Verrill, Castine
Cc: Nickerson, Terri; Schwenn, Molly
Subject: RE: N. Deering

Castine,

Alright, I've got something for you to look at. It's in our cross-share, which you *should* have access to (if not, Terri will):

[\\dhshsa6fsom001\mcr-eph](#)

- and then go into the "North Deering Project" folder.

I created an Excel file, "Portland_final_list_with_tract_data.xls" which has a table of the cancer records, with Tract IDs, and a separate table of Tract demographics (this comes directly from MEGIS).

The final selection are all those addresses within a Portland census tract OR having an unmapped address that was unknown or a PO Box (but appeared to be in Portland). I left those in so you could include the "non-mapped, but in Portland" in the final tallies.

I have to admit that time has not allowed me to really pore over the final locations/data. In the final geocoding process I did do a cross-check with an automated geocoding run, and I resolved a few more of the non-geocoded ones. I think it's pretty clean, and I trust and know that Terri did a good job fixing up the recalcitrant addresses.

It looks to me like there are 3,267 out of 3,398 mapped (96%), for the Portland-assigned addresses. That's pretty good. The remaining 4% are mostly PO Boxes or "Unknowns" with very few having an inscrutable address.

The final source data are in an Access file. I'm guessing you won't want to mess with that.

A project map is in an ArcGIS file, "Portland_project_map.mxd". You might want to take a look at that.

I hope this is enough for you to take it away! Any issues, of course...contact me.

I won't be in the office Monday, I'm away at a conference Tuesday-Thursday. Back on Friday. But then gone on vacation the following week!

So just email...

CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 1:03 PM
To: Paulu, Chris
Subject: RE: N. Deering

That's correct. Thank you so much for working on this with everything else that's going on. I still need to get you the additional county data for the lung data. Sorry for the delay.

From: Paulu, Chris
Sent: Friday, April 04, 2008 1:00 PM
To: Verrill, Castine
Subject: RE: N. Deering

Let me just confirm one thing though: this is Portland only analysis, right? The only reason we have Falmouth, etc., is due to overlap, mistaken zips, and so on. Right?

Okay.

CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 12:48 PM
To: Paulu, Chris
Subject: N. Deering

Hi Chris,

It is crazy to think I only have a week and a half of work left! I know you are busy, but the North Deering concern is something I hate to leave unfinished. I was wondering if you have made progress on the mapping and census tract application for the Portland data. Also, do you have access to the Census 2000 gender/age-specific population files for the Census Tracts within Portland?

Thanks for your help!

Castine

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Cc: [Nickerson, Terri](#); [Schwenn, Molly](#)
Subject: RE: N. Deering
Date: Friday, April 4, 2008 5:11:39 PM

Castine,

Alright, I've got something for you to look at. It's in our cross-share, which you *should* have access to (if not, Terri will):

[\\dhshsa6fsom001\mcr-epht](#)

- and then go into the "North Deering Project" folder.

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I won't be in the office Monday, I'm away at a conference Tuesday-Thursday. Back on Friday. But then gone on vacation the following week!

So just email...

CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 1:03 PM
To: Paulu, Chris
Subject: RE: N. Deering

That's correct. Thank you so much for working on this with everything else that's going on.

I still need to get you the additional county data for the lung data. Sorry for the delay.

From: Paulu, Chris
Sent: Friday, April 04, 2008 1:00 PM
To: Verrill, Castine
Subject: RE: N. Deering

Let me just confirm one thing though: this is Portland only analysis, right? The only reason we have Falmouth, etc., is due to overlap, mistaken zips, and so on. Right?

Okay.

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Subject: N. Deering

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Thanks for your help!
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From: [Verrill, Castine](#)
To: [Paulu, Chris](#)
Subject: RE: N. Deering
Date: Friday, April 4, 2008 1:03:23 PM

That's correct. Thank you so much for working on this with everything else that's going on.

I still need to get you the additional county data for the lung data. Sorry for the delay.

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Sent: Friday, April 04, 2008 1:00 PM
To: Verrill, Castine
Subject: RE: N. Deering

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Okay.

CP

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Subject: N. Deering

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Subject: RE: N. Deering
Date: Friday, April 4, 2008 1:00:28 PM

Let me just confirm one thing though: this is Portland only analysis, right? The only reason we have Falmouth, etc., is due to overlap, mistaken zips, and so on. Right?

Okay.

CP

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Sent: Friday, April 04, 2008 12:48 PM
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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Subject: RE: N. Deering
Date: Friday, April 4, 2008 12:58:41 PM

Hi,

Yeah, I know!

Worse? I'm at a conference most of next week, and then on vacation the week after.

--

So I've been working hard to wrap this up. Almost there.

I think I can get the Census 2000 data too.

CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 12:48 PM
To: Paulu, Chris
Subject: N. Deering

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Cc: [Nickerson, Terri](#)
Subject: RE: N. Deering
Date: Tuesday, May 22, 2007 9:54:18 AM

Yup, sounds good. See you then.

-----Original Message-----

From: Verrill, Castine
Sent: Tuesday, May 22, 2007 9:43 AM
To: Paulu, Chris
Cc: Nickerson, Terri
Subject: RE: N. Deering

Hi Chris,

Thanks for helping us out, especially given your deadline. Would you be available on Thursday at 2 p.m.? Terri is able to log in to Citrix, so we would be meeting in her office.

Thanks,
Castine

-----Original Message-----

From: Paulu, Chris
Sent: Monday, May 21, 2007 2:38 PM
To: Verrill, Castine
Cc: Nickerson, Terri
Subject: RE: N. Deering

Hi Cas,

Sorry I didn't get back to you. I'm good for tomorrow, or Thursday. I have a June 1st manuscript deadline breathing down my neck, and Andy told me to cancel all meetings. But when I explained that this is for the North Deering response, and my objective was just to get *you* started, he was okay with the idea.

So...I just want to make sure that you can both log in to Citrix. I know you have the data assembled. I think we're good to get cookin'. I'll bring some recipe ideas.

I'll suggest 9:30am either tomorrow or Tuesday. But I could do any time after 1pm as well.

Chris

From: Verrill, Castine
Sent: Friday, May 11, 2007 6:06 PM
To: Paulu, Chris
Cc: Schwenn, Molly; Nickerson, Terri
Subject: RE: N. Deering

Hi Chris,

Thank you for looking at the census tracts and identifying additional streets to be included in the analysis. I have pulled a file of all cases in the 04103 zip code as well as the specified streets in the 04102 zip code. We are currently reviewing the file for any duplicate cases. Terri will be taking a look at the file for address standardizing issues and may want to speak with you about how to move forward with the process. I have yet to log into Citrix to make sure ArcGIS runs o.k. - Terri has it on her computer if one of you wants to check into it.

I will be out of town until next Friday, the 18th. Could we meet on Tuesday the 22nd for some ArcGIS training? I'm free all day. I'm also available all day on the 24th if that works better.

Thanks and have a great weekend! Whee :-)

Cas

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Cc: [Nickerson, Terri](#)
Subject: RE: N. Deering
Date: Tuesday, May 22, 2007 9:42:41 AM

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Thanks for helping us out, especially given your deadline. Would you be available on Thursday at 2 p.m.? Terri is able to log in to Citrix, so we would be meeting in her office.

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(207) 287-5190
castine.verrill@maine.gov

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Cc: [Nickerson, Terri](#)
Subject: RE: N. Deering
Date: Monday, May 21, 2007 2:38:18 PM

Hi Cas,

Sorry I didn't get back to you. I'm good for tomorrow, or Thursday. I have a June 1st manuscript deadline breathing down my neck, and Andy told me to cancel all meetings. But when I explained that this is for the North Deering response, and my objective was just to get *you* started, he was okay with the idea.

So...I just want to make sure that you can both log in to Citrix. I know you have the data assembled. I think we're good to get cookin'. I'll bring some recipe ideas.

I'll suggest 9:30am either tomorrow or Tuesday. But I could do any time after 1pm as well.

Chris

From: Verrill, Castine
Sent: Friday, May 11, 2007 6:06 PM
To: Paulu, Chris
Cc: Schwenn, Molly; Nickerson, Terri
Subject: RE: N. Deering

Hi Chris,

Thank you for looking at the census tracts and identifying additional streets to be included in the analysis. I have pulled a file of all cases in the 04103 zip code as well as the specified streets in the 04102 zip code. We are currently reviewing the file for any duplicate cases. Terri will be taking a look at the file for address standardizing issues and may want to speak with you about how to move forward with the process. I have yet to log into Citrix to make sure ArcGIS runs o.k. - Terri has it on her computer if one of you wants to check into it.

I will be out of town until next Friday, the 18th. Could we meet on Tuesday the 22nd for some ArcGIS training? I'm free all day. I'm also available all day on the 24th if that works better.

Thanks and have a great weekend! Whee :-)

Cas

-----Original Message-----

From: Paulu, Chris
Sent: Wednesday, May 09, 2007 12:17 PM
To: Verrill, Castine
Subject: RE: N. Deering

Hi Cas,

I'm going to pretend we're going ahead with this North Deering project. I was looking at the Census Tracts yesterday, and there appears to be a triangular sliver of zipcode 04102 that extends into one of the tracts of interest. To be safe, I think we should add a

selection of those streets to the selection of the main zipcode. Here they are...

Add the following streets from Zipcode 04102:

- Holm Ave
- Birchwood Dr
- Aspen Dr
- Pinecrest

I would do this either by sorting the street names to look for these names and their variants, or just filtering using the name alone (i.e., "holm", "birchwood", etc.). Whatever will have the best chance of catching those streets, basically.

Other than that, the only prep work is to create an Access file that contains all the addresses. Also, log in to Citrix and make sure the ArcGIS stuff runs properly.

Then we can arrange a time to get started. Whee!

CP

From: Verrill, Castine
Sent: Tuesday, May 08, 2007 2:54 PM
To: Paulu, Chris
Cc: Schwenn, Molly; Smith, Andy E.
Subject: N. Deering

There are approximately 1,800 cases to geocode for the 04103 zip code from 1995-2005.

Thanks for your input on the timeline for completing the process.

Castine Verrill, MS

Epidemiologist
Maine Cancer Registry
Maine CDC
(207) 287-5190
castine.verrill@maine.gov

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Subject: RE: N. Deering
Date: Thursday, April 17, 2008 8:23:16 AM

Yeah, I also noticed that it doesn't have a breakdown by sex.
I'll see what the Census has. Let me know if you've already checked.
CP

From: Verrill, Castine
Sent: Tuesday, April 15, 2008 6:31 PM
To: Paulu, Chris
Cc: Schwenn, Molly
Subject: RE: N. Deering

Hi Chris,

I looked at the file referenced below. Thank you for re-linking the databases to include the original data items from the confidential file. You did include 2000 Census population figures by census tract; however, the age groupings are not the same as what we have for our cases. Is it possible to get the population data by 5-year age groups (e.g. 0-4, 5-9, 10-14...85+)?

Castine

Castine Verrill, MS

Epidemiologist

Maine Cancer Registry

MECDC, DHHS

(207) 287-5190

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From: Paulu, Chris
Sent: Friday, April 04, 2008 5:12 PM
To: Verrill, Castine
Cc: Nickerson, Terri; Schwenn, Molly
Subject: RE: N. Deering

Castine,

Alright, I've got something for you to look at. It's in our cross-share, which you *should* have access to (if not, Terri will):

[\\dhhshsa6fsom001\mcr-epht](#)

- and then go into the "North Deering Project" folder.

I created an Excel file, "Portland_final_list_with_tract_data.xls" which has a table of the cancer records, with Tract IDs, and a separate table of Tract demographics (this comes directly from MEGIS).

The final selection are all those addresses within a Portland census tract OR having an unmapped address that was unknown or a PO Box (but appeared to be in Portland). I left those in so you could include the "non-mapped, but in Portland" in the final tallies.

I have to admit that time has not allowed me to really pore over the final locations/data. In the final geocoding process I did do a cross-check with an automated geocoding run, and I resolved a few more of the non-geocoded ones. I think it's pretty clean, and I trust and know that Terri did a good job fixing up the recalcitrant addresses.

It looks to me like there are 3,267 out of 3,398 mapped (96%), for the Portland-assigned addresses. That's pretty good. The remaining 4% are mostly PO Boxes or "Unknowns" with very few having an inscrutable address.

The final source data are in an Access file. I'm guessing you won't want to mess with that.
A project map is in an ArcGIS file, "Portland_project_map.mxd". You might want to take a look at that.

I hope this is enough for you to take it away! Any issues, of course...contact me.
I won't be in the office Monday, I'm away at a conference Tuesday-Thursday. Back on Friday. But then gone on vacation the following week!
So just email...
CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 1:03 PM
To: Paulu, Chris
Subject: RE: N. Deering

That's correct. Thank you so much for working on this with everything else that's going on.
I still need to get you the additional county data for the lung data. Sorry for the delay.

From: Paulu, Chris
Sent: Friday, April 04, 2008 1:00 PM
To: Verrill, Castine
Subject: RE: N. Deering

Let me just confirm one thing though: this is Portland only analysis, right? The only reason we have Falmouth, etc., is due to overlap, mistaken zips, and so on. Right?
Okay.
CP

From: Verrill, Castine
Sent: Friday, April 04, 2008 12:48 PM
To: Paulu, Chris
Subject: N. Deering

Hi Chris,
It is crazy to think I only have a week and a half of work left! I know you are busy, but the North Deering concern is something I hate to leave unfinished. I was wondering if you have made progress on the mapping and census tract application for the Portland data. Also, do you have access to the Census 2000 gender/age-specific population files for the Census Tracts within Portland?

Thanks for your help!

Castine

Castine Verrill, MS

Epidemiologist

Maine Cancer Registry

MECDC, DHHS

(207) 287-5190

castine.verrill@maine.gov

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From: [Smith, Andy E. \(DHHS\)](#)
To: [Verrill, Castine](#)
Subject: RE: North Deering Cancer Concern
Date: Wednesday, February 14, 2007 10:32:33 AM

Keep in mind that FERPA is much more restrictive than HIPAA. The school cannot release any identifiable data from their school records without consent.

- Andy

Andrew E. Smith, SM, ScD
State Toxicologist and Director,
Environmental and Occupational Health Program
Maine Center for Disease Control and Prevention
11 State House Station
Augusta, ME 04333
Tel: (207) 287-5189
Fax: (207) 287-3981
Pager: (207) 759-7156
email: andy.e.smith@maine.gov

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-----Original Message-----

From: Verrill, Castine
Sent: Wednesday, February 14, 2007 9:17 AM
To: Smith, Andy E.
Subject: FW: North Deering Cancer Concern

[Andy,](#)

[On Dec. 18th, we received a letter of concern from the Lyseth School about cancer in children and adults in the North Deering community. Per our normal protocol, we asked them to provide information on the cases \(see bottom of this email\). We have yet to receive any additional information to move forward with this request. Below is the email trail about why this is in the news.](#)

[Castine](#)

-----Original Message-----

From: Verrill, Castine
Sent: Tuesday, February 13, 2007 10:18 AM
To: Mills, Dora A.; Poirier, Barbara
Cc: Schwenn, Molly; Martins, John A; Zukas-Lessard, Chris
Subject: RE: North Deering Cancer Concern

Good morning,

I just spoke with the principal at the Lyseth School in North Deering. The principal was concerned about sending us the requested information on people with cancer and violating their privacy. For this reason, a letter was sent out to the parents of all the children at the Lyseth School to request permission to share the 9 pieces of information we requested. I'm not sure why they didn't just send the letter to the 29 individuals they were concerned about, but as a result there will be more than 29 cases reported to us. One of the parents released the letter to the news.

The principal will send us all the information she receives, including additional cases beyond the 29, since the concern is about the North Deering neighborhood rather than the Lyseth School. We will then evaluate the information according to our routine procedure. We will look at the overall number of cases in North Deering to see if it is higher than expected. We are working with the the City of Portland to get accurate population counts for the neighborhood.

Thank you,
Castine

-----Original Message-----

From: Mills, Dora A.

Sent: Monday, February 12, 2007 6:02 PM

To: Poirier, Barbara

Cc: Schwenn, Molly; Verrill, Castine; Martins, John A; Zukas-Lessard, Chris

Subject: RE: North Deering Cancer Concern

Importance: High

Thank you for the information. I suspect that they interpreted the letter, then, as meaning that they should be recruiting their own reporting system for cancer cases, as apparently that's the tone of the letter - "let us know if you know of anyone with cancer, we'll report it to the Me CDC". I think we thought they knew of 29 cases and we just needed to know whom they are. However, I thought our overall approach is to look at the prevalence/incidence for the geographical area concerned, and then compare that to an expected rate. Is there someone who can call the school principal, now that this is out in the news, and work with them on this? I'm concerned that the process described by the tv stations (asking for cancer cases to be reported to them) will result in a lot of unnecessary fear.

Dora

From: Poirier, Barbara

Sent: Mon 2/12/2007 5:53 PM

To: Mills, Dora A.

Cc: Schwenn, Molly; Verrill, Castine

Subject: FW: North Deering Cancer Concern

Hi Dora, Castine and Molly told me about Kim's call. Said she was asking about Maine cancer prevalence. Castine, at my request, referred Kim to Kip DeSerre who gave Castine an estimated figure of 40-50,000 based on national data. Below is email request Castine sent to the school back in December and has not heard anything since. Sounds like the school may have initiated the letter. B

I'm in DC and out and about, but got a cell phone call from Channel 13/WGME at ~ noon, asking about a letter that went home to parents of the elementary school that wrote us a few weeks ago expressing concerns about high rates of cancer. They quoted the letter, saying it requests parents to call the school nurse to report any cancer cases, and that they

would then report that information to us! I told them to call Dr. Schwenn, that she would know if we had requested the letter, but to my knowledge we had not. I told them that because cancer is a reportable disease, we normally do not ask the public to report to us cases of cancer. I just returned to the hotel and received another call - this one from Channel 8, saying they had heard a report on WGME (must have been the 5:00 news) and heard about this letter and also wondered what our role is in it. With both, I also emphasized that we get a half dozen to a dozen such inquiries every year and that over the past 25-30 years we've been doing this, we've encountered one possible cluster. However, we take all such inquiries seriously, and it takes usually several months to complete our work in answering people's questions (I purposely did not use the word "investigation").

Can you pls give us an update on this situation? Thank you!! Dora

-----Original Message-----

From: Mills, Dora A.

Sent: Tuesday, December 26, 2006 12:40 PM

To: Verrill, Castine

Cc: Schwenn, Molly; Poirier, Barbara; Zukas-Lessard, Chris

Subject: RE: North Deering Cancer Concern

Thank you very much, Castine, for your quick and thorough reply. Please keep me in the loop. Dora

From: Verrill, Castine

Sent: Tue 12/26/2006 10:00 AM

To: 'portej@portlandschools.org'; 'russep@portlandschools.org';
'mcleoj@portlandschools.org'

Cc: Schwenn, Molly; Mills, Dora A.

Subject: North Deering Cancer Concern

Dear All Concerned Parties,

I am writing from the Maine Cancer Registry, a program within the Maine Center for Disease Control and Prevention (formerly the Bureau of Health). Dr. Dora Mills forwarded your letter of concern about cancer in the neighborhood of North Deering. The Maine Cancer Registry takes such concerns seriously and has an established protocol for evaluating these concerns. The first step is to gather information on the 29 cancer cases that have been identified. For each case, we will need to know the following:

1. Name
2. Gender
3. Type of cancer (breast, lung, leukemia, etc.)
4. Year the cancer was diagnosed
5. The person's age when the cancer was diagnosed
6. Where the person was living at the time of diagnosis
7. How long the person had lived at this residence
8. Whether the person is still alive
9. Hospital where the person was diagnosed or treated (if known)

Once this information is received, we will attempt to verify the cases in our database. We will also search for additional cases diagnosed in residents of North Deering. If there are specific environmental concerns, please notify us of these as well. This information will then be reviewed by our Cancer Inquiry Team to determine the appropriate next steps.

The case information above can be shared with me over the phone, fax, or via mail. Please

refrain from sending the information by email, as it is not a secure means for transmitting confidential data.

Thank you for bringing this concern to our attention. Please contact me with any questions.

Sincerely,

Castine Verrill, MS

Epidemiologist

Maine Cancer Registry

Maine CDC

286 Water Street, 4th Floor

11 State House Station

Augusta, ME 04333

Phone: (207) 287-5190

Fax: (207) 287-4631

castine.verrill@maine.gov

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From: [Paulu, Chris](#)
To: [Verrill, Castine](#); [Smith, Andy E. \(DHHS\)](#)
Cc: [Schwenn, Molly](#)
Subject: RE: Pop of North Deering
Date: Friday, February 23, 2007 5:16:01 PM
Attachments: [North Deering-tracts and zips.doc](#)

Hi,

It will be no problem to geocode this ourselves, and I see little point in involving MEGIS - unless I happen to be on vacation. If you have the GIS software on hand, this would be a good opportunity to set it up for geocoding and use it (I can do that for you).

--

It looks reasonable to select by zipcode in this instance. This will capture the identified census tracts, plus another that ought to be included (see attached). There is a small wedge of one tract that is not part of 04103. We should be aware of that, but we'll have a better idea of whether or not it matters once the addresses are mapped.

--

CP

-----Original Message-----

From: Verrill, Castine
Sent: Friday, February 23, 2007 4:15 PM
To: Smith, Andy E.; Paulu, Chris
Cc: Schwenn, Molly
Subject: FW: Pop of North Deering

Hi Andy and Chris,

As you may have heard, we received a request to evaluate cancer and the "environmental integrity" of North Deering. North Deering is a neighborhood located within the 04103 zip code of Portland, along with many other communities. North Deering is not a town, nor does it have a specific zip code, so there is no easy way to identify the cases in our database or establish an underlying population.

The City of Portland has identified the boundaries of North Deering and matched the boundaries to two census tracts (please see email trail below). The population data of the census tracts within the zip code are attached in the Excel file. Since we don't have census tract information in our databases yet, it looks like we will have to geocode the cases in the 04103 zip code and see how many are located in the two census tracts associated with North Deering. I took a quick look at the data and most have physical street addresses.

I'd like to get your input on whether we should work with MEGIS to geocode and assign census tracts to this limited data set. We would need to proceed in a relatively short time frame. What are your thoughts?

Thanks!

Castine Verrill, MS
Epidemiologist

Maine Cancer Registry
Maine CDC
(207) 287-5190
castine.verrill@maine.gov

-----Original Message-----

From: TOHO SOMA [<mailto:TSOMA@portlandmaine.gov>]
Sent: Thursday, February 15, 2007 1:31 PM
To: Verrill, Castine
Subject: RE: Pop of North Deering

Hi again-

Attached is the updated spreadsheet, listing population for each census tract in the 04103 zip code by 5-year age group for 2000 and 1990. As you can see, the zip code area and census tracts are do not align perfectly. Also, population by zip code was not captured in 1990.

Hope this helps.
Toho

Toho Soma, MPH
Public Health Information Program Coordinator Epidemiology and Emergency Preparedness Program Public Health
Division, City of Portland
389 Congress Street, 3rd Floor
Portland, ME 04101
tsoma@portlandmaine.gov
T: 207-874-8787 / F: 207-874-8913

>>> "Verrill, Castine" <Castine.Verrill@maine.gov> 2/15/2007 11:00:55 AM >>>

Hi Toho,

Thank you very much for the population data. If possible, could I get the population for each 5-year age group (0-4, 5-9, 10-14.... 85+) for the two census tracts you identified? Also, within the zip code area of 04103 are there other census tracts? If so, would you be able to provide the population distribution for these census tracts separately?

We may only be able to look at our data by zip code (rather than census tract) and would need the corresponding population figures.

Thanks for your help. Please call if you have any questions.

Castine

-----Original Message-----

From: TOHO SOMA [<mailto:TSOMA@portlandmaine.gov>]
Sent: Thursday, February 15, 2007 10:21 AM
To: Verrill, Castine; Michael Russell
Cc: Schwenn, Molly; Bethany Sanborn
Subject: RE: Pop of North Deering

Good morning-

I've investigated various sources to try and pinpoint the population of North Deering. I first went to the North Deering Neighborhood Association's website, where they had a map of what they considered to

be North Deering's boundaries. I then concluded that they were based on the US Census Bureau's census tracts (#21.01 and 22 in Cumberland County).

Here's the glitch--census tract populations are only counted every 10 years, so I could only get population data for 1990 and 2000. Fortunately, they also enumerate by age. The attached spreadsheet shows the population for each age group requested for the two census years. Annual population estimates are only done down to the city level, and even then, not separated by age group.

If you have any questions or if I can be of any further assistance, please let me know.

Thanks,

Toho

Toho Soma, MPH
Public Health Information Program Coordinator
Epidemiology and Emergency Preparedness Program
Public Health Division, City of Portland
389 Congress Street, 3rd Floor
Portland, ME 04101
tsoma@portlandmaine.gov
T: 207-874-8787 / F: 207-874-8913

Zipcode 04103, highlighted in **blue**

Census tracts in **purple**

Towns in **red**

From: [Paulu, Chris](#)
To: [Verrill, Castine](#)
Subject: RE: Pop of North Deering
Date: Monday, February 26, 2007 9:44:16 AM

For this size dataset, that will work fine.

CP

-----Original Message-----

From: Verrill, Castine
Sent: Monday, February 26, 2007 9:34 AM
To: Paulu, Chris
Subject: RE: Pop of North Deering

Hi Chris,

I'm glad to hear that we can do this ourselves. I have access to ArcView 9.1 through the Citrix server. Will this work for our needs?

Thanks,
Castine

-----Original Message-----

From: Paulu, Chris
Sent: Friday, February 23, 2007 5:11 PM
To: Verrill, Castine; Smith, Andy E.
Cc: Schwenn, Molly
Subject: RE: Pop of North Deering

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Cc: Schwenn, Molly
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Thanks!

Castine Verrill, MS
Epidemiologist
Maine Cancer Registry
Maine CDC
(207) 287-5190
castine.verrill@maine.gov

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To: Verrill, Castine
Subject: RE: Pop of North Deering

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Toho Soma, MPH
Public Health Information Program Coordinator Epidemiology and Emergency Preparedness Program Public Health
Division, City of Portland
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T: 207-874-8787 / F: 207-874-8913

>>> "Verrill, Castine" <Castine.Verrill@maine.gov> 2/15/2007 11:00:55 AM >>>

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We may only be able to look at our data by zip code (rather than census tract) and would need the corresponding population figures.

Thanks for your help. Please call if you have any questions.

Castine

-----Original Message-----

From: TOHO SOMA [<mailto:TSOMA@portlandmaine.gov>]

Sent: Thursday, February 15, 2007 10:21 AM

To: Verrill, Castine; Michael Russell

Cc: Schwenn, Molly; Bethany Sanborn

Subject: RE: Pop of North Deering

Good morning-

I've investigated various sources to try and pinpoint the population of North Deering. I first went to the North Deering Neighborhood Association's website, where they had a map of what they considered to be North Deering's boundaries. I then concluded that they were based on the US Census Bureau's census tracts (#21.01 and 22 in Cumberland County).

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If you have any questions or if I can be of any further assistance, please let me know.

Thanks,

Toho

Toho Soma, MPH
Public Health Information Program Coordinator
Epidemiology and Emergency Preparedness Program
Public Health Division, City of Portland
389 Congress Street, 3rd Floor
Portland, ME 04101
tsoma@portlandmaine.gov
T: 207-874-8787 / F: 207-874-8913

From: [Nickerson, Terri](#)
To: [Paulu, Chris](#)
Subject: RE: Portland #?
Date: Monday, October 22, 2007 2:24:42 PM

Chris,
The number of cases currently being looked at for the North Deering project is 4036. The years are from 1995 through 2004.

Terri Nickerson
Maine Cancer Registry
207-287-5272 phone
207-287-4631 fax
1-800-606-0215 TTY

From: Paulu, Chris
Sent: Monday, October 22, 2007 1:07 PM
To: Nickerson, Terri
Subject: Portland #?

Hi Terri,
What is the total number of records that are being geocoded for Portland (or Portland/Falmouth?), and what years are those from?
I just want to include this task in our EPHT progress report.
Thanks!
Chris

From: [Decker, Kathy L.](#)
To: [Smith, Andy E. \(DHHS\)](#); [Paulu, Chris](#)
Subject: RE: cancer cluster
Date: Friday, September 23, 2011 12:08:19 PM
Attachments: [NDeering_07252008.xls](#)
[NDeering_DataReport032009.doc](#)
[NDeering_rprtble.xls](#)

Here's the last analysis I completed and what looks like a mini report on the analysis.

From: Smith, Andy E. (DHHS)
Sent: Friday, September 23, 2011 11:49 AM
To: Paulu, Chris
Cc: Decker, Kathy
Subject: RE: cancer cluster

How much time did you spend on this. Kathy, can you give me a status on this, and how much time you have spent on the Deering Oaks cancer cluster.

Andrew E. Smith, SM, ScD
State Toxicologist
Director, Environmental and Occupational Health Programs
Maine Center for Disease Control and Prevention
Department of Health and Human Services
11 State House Station
Augusta, ME 04333
Tel: (207) 287-5189
Fax: (207) 287-3981

From: Paulu, Chris
Sent: Friday, September 23, 2011 11:48 AM
To: Smith, Andy E. (DHHS)
Subject: RE: cancer cluster

Yup that's true. Molly and Kathy might still be working on that. I helped with some geocoding and census tract assignment. I don't know any more.

From: Smith, Andy E. (DHHS)
Sent: Friday, September 23, 2011 11:45 AM
To: Paulu, Chris
Subject: cancer cluster

Didn't you assist with some Portland area cluster investigation (Deering oakes?) and didn't that take some time? What was found?

Andrew E. Smith, SM, ScD
State Toxicologist
Director, Environmental and Occupational Health Programs
Maine Center for Disease Control and Prevention

Department of Health and Human Services
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NORTH DEERING NEIGHBORHOOD ANALYSIS

Introduction

In MONTH YEAR, a group of concerned citizens contacted the Maine Cancer Registry with concerns regarding the number of persons diagnosed with cancer in Portland's North Deering neighborhood. The following information is based on analysis completed to assist with the investigation into the number of incident cancer cases in the North Deering neighborhood. Of particular interest was the suspected number of incident cases found in the youth/young adult population. The intent of the analysis was to see whether the number of incident cancer cases in the North Deering neighborhood differed from what you would expect to find in a Maine population.

Methods

Demographic information on the cases identified by the concerned citizens was procured and cross checked with the Maine Cancer Registry tumor database (IMPAC) for accuracy. There were 3,398 incident tumor cases identified by the Maine Cancer Registry for Portland during the time period of 1995-2004 using IMPAC. Cumberland County and Maine State cases counts were retrieved using SEER Stat software.

Portland case addresses were geocoded according to their latitudinal and longitudinal coordinates using XXX software. Of the 3,398 tumor cases, 136 (4.0%) were not geocoded due to either a missing address or post office box listing and excluded from the analysis. Using GIS software, cases were then categorized and coded into one of the Portland Census tracts.¹

Census 2000 data were used to estimate the population of North Deering and Portland.¹ Post-2000 and inter-censal estimates were used for Cumberland County and Maine State estimates (1995-1999 and 2000-2004, respectively).

Cases were stratified by geography, age at diagnosis, and selected major cancer site. Four strata were constructed for the analysis; North Deering, Portland (excluding North Deering hereafter referred to as just "Portland"), Cumberland County, and Maine State. Two census tracts (23005002101 and 23005002200) were identified as belonging to the North Deering neighborhood. The remaining seventeen census tracts were identified as belonging to the strata named Portland. Age groups used in the analysis were 0-19 years, 20-44 years, 45-64 years, 65-74 years, 75-84 years, and 85 plus years. Cancers were categorized according to the SEER incident cancer site recode definitions.²

In the youngest age group, 0-19 years, cancers were grouped and analyzed by the following categories; all cancers, lymphocytic leukemia, acute lymphocytic leukemia, myeloid and monocytic leukemia, other leukemia, Hodgkin lymphoma, Non-Hodgkin

¹ Portland Census tracts: 23005000100, 23005000200, 23005000300, 23005000500, 23005000600, 23005001000, 23005001100, 23005001200, 23005001300, 23005001500, 23005001700, 23005001800, 23005001900, 23005002001, 23005002002, 23005002101, 23005002102, 23005002200, 23005002300.

lymphoma, and brain cancer. All of the other age groups had incident cancer cases analyzed according to the following groups; all cancers, lung, colon, breast (female only), and prostate cancer. Three additional cancer sites were analyzed for the 20-44 year old age group; melanoma, thyroid, and kidney.

Rates and accompanying 95% confidence intervals were calculated using SAS software for the North Deering and Portland analysis³. The expected number of cases was calculated for North Deering based on each of the three remaining strata, i.e. Portland, Cumberland County and Maine State.

Results

Between 1995 and 2004 there were 10 cases of incident malignant cancers (insitu included for brain cancers) found in persons 0-19 years of age in the North Deering neighborhood, 19 cases in Portland, 139 cases in Cumberland County, and 609 cases in total for Maine (Table 1). Of the ten cases found in North Deering, eight are accounted for in the individual incident site categories presented next. One case each of respiratory and endocrine site cancers were not included in the individual site analysis (data not shown).

In persons aged 0-19 years there were no differences in the observed number of cases when using Cumberland County or Maine as the basis for acute lymphocytic, and brain cancer (Tables 2-3). North Deering experienced higher than the expected number of cases of myeloid and monocytic leukemia (17.0 per 100,000), and Hodgkin lymphoma (17.0 per 100,000) compared to Cumberland County (1.2 and 2.3 per 100,000, respectively) and Maine (1.0 and 1.6 per 100,000, respectively)(Tables 4-5). There were no cases of other leukemia or Non-Hodgkin lymphoma reported in North Deering during the same time period (1995-2004)(Table 6).

For the same time period, there were 53 cases of incident malignant cancers found in persons 20-44 years of age in North Deering neighborhood, 198 cases in Portland, 1,061 cases in Cumberland County, and 5,081 in total for Maine (Table 7). Of the 53 cases found in North Deering, 31 are accounted for in the individual incident site categories presented. Of the twenty-two incident cancers not compared in the following analysis, 6 were lymphoma, 3 cervix or ovary, 2 oral cavity, 3 digestive, 1 respiratory, 1 brain, 2 testis, 1 invalid (data not shown)(*I know I'm missing 3 cancers in the group*).

In persons aged 20-44 years there were no differences in the observed number of cases when using Portland, Cumberland County or Maine as the basis for lung, colon, breast, and prostate cancers (Tables 8-11). North Deering experienced higher than the expected number of cases of melanoma (26.2 per 100,000) compared to Portland, Cumberland County, and Maine (7.2, 10.8, and 12.4 per 100,000, respectively)(Table 12). North Deering experienced a higher number of thyroid and kidney cancers (17.5 and 8.7 per 100,000, respectively) compared to Portland (6.0 and 1.8 per 100,000, respectively)(Tables 13-14). There were no observed differences in the expected number of thyroid or kidney cancer in North Deering compared to Cumberland County or Maine.

For the same time period in persons aged 45-64 years, there were 162 cases of incident malignant cancers in North Deering neighborhood, 705 cases in Portland, 4,386 cases in Cumberland County, and 23,016 in total for Maine (Table 15). Of the 162 incident cases in North Deering, 94 are accounted for in the site specific analysis.

There were no differences in the observed number of incident cases of lung, colon, and prostate cancer in North Deering compared to Portland, Cumberland County, or Maine (Tables 16-17, 19). The number of breast cancer cases was higher than expected in North Deering (340.4 per 100,000) compared to Portland (251.1 per 100,000), Cumberland County (279.9 per 100,000), and Maine (262.0 per 100,000)(Table 18).

In persons aged 65-84 years there were 328 cases of incident malignant cancers in the North Deering neighborhood, 1,212 cases in Portland, 6,876 cases in Cumberland County, and 38,593 cases in Maine (Table 20). Of the 328 incident cases in North Deering, 180 are accounted for in the site specific analysis.

No differences were observed in the number of incident cases of colon cancer in North Deering compared to Portland, Cumberland County, and Maine (Table 22). North Deering had lower than expected numbers of lung cancer (384.1 per 100,000) compared to Portland (469.6 per 100,000), Cumberland County (427.7 per 100,000), and Maine (443.4 per 100,000)(Table 21). The number of incident cases of prostate cancer were also lower in North Deering (705.7 per 100,000) compared to Cumberland County (806.4 per 100,000) and Maine (980.6 per 100,000)(Table 24). The number of newly identified breast cancers in North Deering (456.8 per 100,000) were higher when compared to Portland (371.0 per 100,000)(Table 23).

In persons aged 85 years or more, there were 89 cases of incident malignant cancers in the North Deering neighborhood, 248 cases in Portland, 1,171 cases in Cumberland County, and 5,635 cases in Maine from 1995 to 2004 (Table 25). Of the 89 incident cases identified, 42 are accounted for in the site specific analysis.

There were no differences in the observed number of cases of breast cancer in North Deering compared to Portland, Cumberland County, or Maine (Table 28). There were fewer cases of colon cancer in North Deering (288.2 per 100,000) compared to Portland (456.2 per 100,000)(Table 27). In North Deering there were higher than expected numbers of lung cancer (432.2 per 100,000) compared to Portland (256.7 per 100,000), Cumberland County (320.2 per 100,000), and Maine (306.9 per 100,000)(Table 26). Prostate cancer was higher in North Deering (1,020.4 per 100,000) compared to Portland (393.0 per 100,000)(Table 29).

Limitations

There are a few limitations for this analysis. First, the small number of cases identified in the North Deering neighborhood limits the precision of our estimates. Additionally, we

identified that of the 136 cases not geocoded and excluded from the analysis 132 were malignant tumors, therefore we do not know if they were missing at random across both site and geography (North Deering versus Portland)(data not shown). In the analysis we identified a few cancer sites that only differed between the North Deering neighborhood and Portland such as, prostate cancer in 65-84 and 85 plus years of age, breast cancer 65-84 years of age, thyroid and kidney cancer in 20-44 year olds. Each of these may have been affected by the missing cases. For example, in the missing cases we identified 10 prostate cancers in men 45-64 years of age, if only 1 of those cases is located in North Deering then the difference in observed numbers between North Deering and Portland goes away (data not shown). This demonstrates the instability of working with small numbers and the effect even small shifts in unknowns may have on the ultimate analysis.

Finally, the intent of this report is to present findings of base analysis only. The report is not a complete description of the cancer types and their relationship to or identification of potential environmental exposures.

¹ Census Bureau. American Factfinder Census 2000 dataset. Available URL: <http://www.census.gov>.

² National Cancer Institute (NCI), Surveillance Epidemiology and End Results (SEER) Incidence site recode. Available at: <http://seer.cancer.gov/siterecode/index.html>

³ SAS Institute, Inc. (2002-2003). *SAS Release 9.1.3*. Cary, NC.

From: [REDACTED]
To: [Schwenn, Molly](#)
Subject: Re: Children's cancer rates
Date: Thursday, August 30, 2012 8:31:45 PM

Thank you for getting back to me so quickly. I'm available anytime on Friday. My number is [REDACTED]

Sent from my iPhone

On Aug 29, 2012, at 5:42 PM, "Schwenn, Molly" <Molly.Sc.Schwenn@maine.gov> wrote:

> Dear [REDACTED]
>
> Chris Zukas forwarded your inquiry to me. I will be happy to speak with you about the findings of the North Deering neighborhood cancer evaluation. I have just located the statistical work done 2008 - 2009 and would like to review it before we talk. No additional work has been done since 2009. I will also review data produced by the first epidemiologist who worked with the North Deering data. She no longer works at the Cancer Registry nor in Maine, since 2008, so she is not available.
>
> If you could give me your phone number and some times on Friday (8/31) or next week that you are available, I will call you. Let me know if you prefer that I give you a time to call me at the Cancer Registry.
>
> Sincerely,
>
> Molly Schwenn, M.D.
> Director, Maine Cancer Registry
> Division of Population Health
>
>
> ----- Original Message -----
> From: [REDACTED]
> Sent: Tuesday, August 28, 2012 11:18 AM
> To: Zukas, Christine
> Subject: Children's cancer rates
>
> Hi-
>
> A few years ago I spoke to the woman who was our state health expert regarding a news report on higher childhood cancer rates in the north deering neighborhood in Portland. I live in [REDACTED] and was wondering if you have any further studies that have been done on this issue. I now have [REDACTED] and this really concerns me. Is there anyone that I can speak with about this?
>
> [REDACTED]
>
> Sent from my iPhone

From: [Zukas, Christine](#)
To: [REDACTED]
Cc: [Schwenn, Molly](#)
Subject: Re: Children's cancer rates
Date: Tuesday, August 28, 2012 11:21:58 AM

Dear [REDACTED]

I have copied Dr. Schween of our Division of Population Health. You can expect to hear back from her.

Thank you for contacting us.
Chris

----- Original Message -----

From: [REDACTED]
Sent: Tuesday, August 28, 2012 11:18 AM
To: Zukas, Christine
Subject: Children's cancer rates

Hi-

A few years ago I spoke to the woman who was our state health expert regarding a news report on higher childhood cancer rates in the north deering neighborhood in Portland. I live in [REDACTED] and was wondering if you have any further studies that have been done on this issue. I now have [REDACTED] and this really concerns me. Is there anyone that I can speak with about this?

[REDACTED]

Sent from my iPhone

From: [REDACTED]
To: [Schwenn, Molly](#)
Subject: Re: Children's cancer rates
Date: Tuesday, September 4, 2012 8:47:27 PM

Hi Molly-

I'm sorry I've missed your calls. Tomorrow afternoon would be a good time to call, after 3. You can also try my [REDACTED]

Thank you,
[REDACTED]

Sent from my iPhone

On Aug 31, 2012, at 5:03 PM, "Schwenn, Molly" <Molly.Sc.Schwenn@maine.gov> wrote:

> I just called and have missed you but I plan to try again before I go home. If that fails, I will talk with you on Tuesday.

>

> Molly Schwenn

> Maine Cancer Registry

>

> -----Original Message-----

> From: [REDACTED]

> Sent: Thursday, August 30, 2012 8:31 PM

> To: Schwenn, Molly

> Subject: Re: Children's cancer rates

>

> Thank you for getting back to me so quickly. I'm available anytime on Friday. My number is [REDACTED].

>

> Sent from my iPhone

>

> On Aug 29, 2012, at 5:42 PM, "Schwenn, Molly" <Molly.Sc.Schwenn@maine.gov> wrote:

>

>> Dear [REDACTED],

>>

>> Chris Zukas forwarded your inquiry to me. I will be happy to speak with you about the findings of the North Deering neighborhood cancer evaluation. I have just located the statistical work done 2008 - 2009 and would like to review it before we talk. No additional work has been done since 2009. I will also review data produced by the first epidemiologist who worked with the North Deering data. She no longer works at the Cancer Registry nor in Maine, since 2008, so she is not available.

>>

>> If you could give me your phone number and some times on Friday (8/31) or next week that you are available, I will call you. Let me know if you prefer that I give you a time to call me at the Cancer Registry.

>>

>> Sincerely,

>>

>> Molly Schwenn, M.D.

>> Director, Maine Cancer Registry

>> Division of Population Health

>>

>>

>> ----- Original Message -----

>> From: [REDACTED]

>> Sent: Tuesday, August 28, 2012 11:18 AM

>> To: Zukas, Christine

>> Subject: Children's cancer rates

>>

>> Hi-

>>

>> A few years ago I spoke to the woman who was our state health expert regarding a news report on higher childhood cancer rates in the north deering neighborhood in Portland. I live in [REDACTED] and was wondering if you have any further studies that have been done on this issue. I now have [REDACTED] and this really concerns me. Is there anyone that I can speak with about this?

>>

>> [REDACTED]

>>

>> Sent from my iPhone

From: [Molly Schwenn](#)
To: [Kathy Decker](#)
Cc: [Meyer, Katie](#); [Paulu, Chris](#); [Schwenn, Molly](#)
Subject: Re: North Deering Analysis
Date: Monday, March 16, 2009 10:34:06 AM

Kathy,

Thanks for pulling the North Deering data together. I can be available for a meeting on 3/31 at either 3:30 or 4 PM; I prefer 3:30.

I haven't reviewed all of the data yet but am having trouble understanding some of the Expected numbers, or perhaps how you arrived at them. Also, is the Probability related to the crude rates or to Observed vs. Expected number? Please review before we meet next week.

I'm sorry that I requested having all the tables separate; I don't think I knew what that would mean. I should have asked you to group them on separate pages by age.

Molly

On Sun, Mar 15, 2009 at 4:20 PM, Kathy Decker <kathyd0120@gmail.com> wrote:

Dear all,

I am attaching a draft report of the analysis to date for North Deering investigation. The intent of this report is just to consolidate the findings in terms of comparison across geography. The intent is not to talk about or discuss 'why' we may be finding differences or identify any environmental risk factors or exposures. You will find two documents one a .doc that is the report body (very short only 4 pages) and an .xls that includes all the tables referenced in the report.

At our last meeting I was asked to look at the distribution of 0-19 year old cancers, represented here:

**Number of malignant cancer cases, age 0-19 years, by year
of diagnosis, 1995-2004**

Year	North Deering	Portland
1995	0	1
1996	0	4
1997	0	3
1998	2	7
1999	2	3
2000	3	4
2001	1	2
2002	1	2
2003	0	2
2004	1	1
	10	29

I was also asked to provide the comparison of Portland excluding North Deering. The report presents information on the comparison of Portland excluding North Deering. I was also asked to calculate the rates of melanoma, thyroid, and kidney cancers in the 20-44 year old

age group. This is also included in the report. Lastly, I was asked to pull the myeloid and monocytic leukemia counts for 2005 & 2006; Maine 4 in 2005 and 5 in 2006, Cumberland County 0 in 2005 and 2 in 2006 (these were pulled from SeerStat, so not sure if the 2006 data is up to date).

I know it's been quite some time but I believe these were all my tasks. Finally, Molly has asked me to set up a time for us to meet again and discuss if we need anything further to make some kind of statement. She needs to meet with the North Deering folks and wants your input on how to present the information collected.

Anyway, I would like to set up a meeting with all of us for Tuesday, March 31st at 3:30 or 4 pm if you are available. Please respond and let me know your availability. Thank you and let me know if you have any questions.

Regards,
Kathy
on

--

molly schwenn

From: [Decker, Kathy](#)
To: [Smith, Andy E. \(DHHS\)](#); [Anderson, Norman](#)
Subject: Week ending 9/23/2011
Date: Monday, September 26, 2011 9:15:07 AM

Dear Andy & Norm,

I have bolded a couple of sections that haven't received follow-up. Here's what I've been up to:

Carbon Monoxide

Active:

a. CO morbidity analysis/manuscript (Deadline: Dec. 2011)

My plan is to begin manuscript writing this Friday. I'll contact Doug this week and set up a time to discuss.

b. CO Surveillance

Met with our CO surveillance group to review CO cases. We had two intentional CO cases and several non case elevated COHgb. Also, requesting additional information from hospital for another possible case. (Deadline: Ongoing)

c. CO Evaluation

We have not received medical record information from CMMC for the non matched CO records. If we do not hear anything this week, I'll contact Ms. Tilley (record disbursement coordinator) and find out when we might expect to receive the requested information. (Deadline (for completeness analysis): Sept. 2, 2011)

d. National Surveillance

I created the datasets for submission to the national portal for CO Hospitalization/ED Visits. (Deadline for data submission: 10/3-7 /2011)

b. CO COGNOS

Nothing new to report.

c. Protocol

Nothing new to report.

c. Metadata

Prepared and submitted metadata for the national portal for CO ED Visits (2008 & 2009) and CO Hosp (2008). This week has been all about metadata! (Deadline: Sept.23,2011)

c. CO Occupational

Nothing new to report.

Birth Outcomes/Vital Stats

Active:

A. Premade reports

Do we still want premade reports updated for BO? Nothing new to report. ????

b. Participate in CWG

The group met at 8:30 am Monday during the Atlanta Conference. I was not able to attend but followed up with Steve McDonald. He indicated that I didn't really miss anything. He did talk about the progress from the Mortality subgroup related to the heat related mortality measures. (Deadline: Ongoing)

c. Community Mapping Project Test

I had intended to follow-up with Tom Talbot during the meeting and talk about the geospatial project for birth outcomes. I was not able to do this. (Deadline: Spring, 2012)

Birth Defects

Active:

a. Prepare national metadata

I created metadata for the 8 birth defect files we will be sending covering the years 2008 & 2009 and submitted (resubmitted). As you know there was some confusion (I think not clear instruction from the national folks) in regard to data that didn't currently exist on the national portal. Anyway, it is all cleared up and apparently we are ahead of the game in that we have our MCN's from last year still intact. (Deadline: Sept. 23, 2011)

b. Prepare national datasets.

I have started preparation of the 8 birth defect data files. I should finish early next week.

(Deadline: Oct. 3-7, 2011)

Cancer

Sent A. Smith time estimates for portion of work on the North Deering neighborhood analysis for the MCR.

Supervisory

-

Active:

- a. Catch up on MFR's for USM.

Spent some time looking for the new year financials on the USM peoplesoft system. They do not have the new cooperative agreement information on the web portal at this time. I have contacted appropriate personnel and asked for follow-up. I go in and reconcile periodically to ensure that they are not including charges not related to our contract. I wanted to see if any of the travel charges had shown up as yet. (Deadline: Ongoing)

- c. Supervise RAI

Nothing new to report.

Overarching/Misc

-

Active:

- a. Contract/Financial

Nothing new to report.

- c. COGNOS/Portal**

Still waiting for comment on the security information sheet developed for cancer secure data.

- d. Water Testing Analysis

Nothing new to report.

- e. BRFS

I'll be meeting with the BRFSS Planning workgroup next week. At this meeting we will vote on what questions to include and where they will be located on the 2012 BRFSS.

f. Surveillance

I have reviewed the MHLPPS surveillance plan edits from E. Frohberg. Scheduled a meeting to discuss and move forward for Monday, Sept. 26th w/Eric & Shuqin. We finalize the table of measures and finish up some of the writing. S. Xing will prepare detailed analysis plans for each measure to be added to the plan. (Deadline: November, 2011)

g. Planning

Nothing new to report.

That's about it for now.

Respectfully submitted,
Kathy

From: [May, Alfred](#)
To: [Smith, Andy E. \(DHHS\)](#)
Subject: Work Week: May 19 -23
Date: Thursday, May 22, 2008 2:36:31 PM

Week of May 19-23, 2008

On my plate

1. Child Lead
 - a. Monthly Team Meetings (discussion as it relates to ABLES as well as PRA I's time being used for data follow-up)
2. EPHT
 - a. Coordinate projects; timeline; track completion.
 - b. Technical Advisory Groups
 - c. AVR Tool Assessment
 - d. EPHT 101
 - e. Workgroups
 - i. PMO
 - ii. PAVR
 - iii. CWG
 - f. Reports (quarterly, interim and annual)
 - g. Budget (Add 12/07)
 - h. Node Work Group (Add 11/07)
2. OH/ABLES
 - a. Coordination with Labor
 - i. Monthly Data Workgroup meeting
 - b. CSTE National OH Workgroup
 - c. CSTE OH Indicators
 - d. BRFSS: Workers Comp Module
 - e. Maine OH Planning; disease surveillance
 - f. ABLES Program coordination
3. EOHP
 - a. Coordination of meetings; office logistics.

Ongoing Issues

1. Child Lead: nothing this week
2. EPHT
 - a. Project Coordination/Tracking: meetings on Maine Portal; reviewed User Insights reports and made notes of key issues—can be discussed with group at next meeting. I need to lay out the work involved to complete the content writing for the portal pages and determine who will be responsible for the final editing and placing in the portal/web pages. Jeremy had stated that he used Visual Basic for setting up the pages, but since we have DreamWeaver, we could consider it. If so, I wonder if we should get some training.
 - b. TAGs: meeting with MCR: need to follow up with Rene on IT support for Molly's VA Togus Data project (NIST security); Cancer cluster North Deering Report—Kathy is now providing contract support for MCR to complete the analysis and report for this project; Chris will provide consult to Kathy on the data that he prepared; one topic for our TAG to work on this next year is a means to

better automate the response to data requests and cluster issues—scope out from August 2008 to March 2009, then budget it in our Interim Report in April 2009; Chris mentioned that the RIF tool may provide some great mapping capabilities for cancer. MCR website needs to be updated. Andy showed the Washington EPHT portal as a sample of what Maine is considering. Webinar on EPHT-Cancer Registries provided some background on EPHT and provided how data may be used, mapping and how the data sharing will be done via clear agreements between CDC and NCI. There was some comments by Cancer Program participants about the amount of work being asked from their minimal staffs for state based data, confidentiality, and suppression. ME-CDC GIS User group: wrote up and disseminated notes from May 8 meeting.

- c. AVR: Need to write up newsletter update or have meeting in June for AVR group.
- d. EPHT 101: nothing this week.
- e. Work Groups: **PMO Outreach 5/20:** participated on call where New Hampshire and Florida shared some of the activities they have planned for the outreach; completed draft of Maine outreach plan, but after discussions on portal design and soft launch, will review/change and have for review next week. **PAVR Portal Team:** conference call with co-chair to determine projects completed and next steps for this year; interacted with Alex (CDC) and Marni (WI) on how our team could provide more resources for states as they develop the look and feel of their portals (see email that I forwarded with Alex's response). **PMO Messaging:** sent drafts of messages to Eric, Chris and Kathy for review and comment; will do review of each CWG message and provide feedback to PMO; received comments from Eric on arsenic/water.
- f. Reports: none this week. Review Risk Communication: we need to develop an ongoing plan and timeline for moving forward on this from now to 2010.
- g. Budget: when Kathy's HP IPAQ arrives, asked Chad to clean up the remainder of the computer components in the Epidemiologist's office; completed airline and shuttle arrangements for travel to Denver CSTE.
- h. Node: it looks like Rene, Lisa and I need to decide who and what times we will cover for this training next week. We should have a short meeting next week prior to the training to know what our data stream is (current is Eric, Tina and Child Lead).

3. Occupational Health

- a. Labor: missed PDAG meeting this week for portal meeting

OH funding proposal: Leslie is completing the draft MOU, utilizing some of the writing in our proposal and should have something for us next week.

- b. CSTE National OH WG: received agenda for OH meeting at Denver for that Sunday June 8.

- c. CSTE OH Indicators: new update on How to Guide has been distributed for getting 2005 data indicators in by June 30; I have cancer data; need to get hospitalization data done this month as well as mortality data update from Brenda.
- d. BRFSS Workers Compensation: nothing new; survey ongoing.
- e. ME OH Planning: working on three month planning documents.
- f. ABLES: report summary sheets: in week ending 5/17, we had 49 reports via mail/fax/email and 83 reports via ELR. Mayo Labs accounted for 41 and 64 of these, respectively. No response on my quick analysis of ABLES time trends of reports by year and BLL. Belinda and I worked on a coding exercise for the ABLES meeting and submitted it; I also requested and received last year's exercise for Belinda and I to work on, so that we can get better at coding. With Belinda sick on Monday and out on Wednesday, ran the NEDSS reports.

4. EOHP

- a. Meeting planning: no meeting this week; moving forward on EPHT and OH planning documents;
- b. Travel: CSTE Denver—finalize travel arrangements.
- c. Emails/Business: updates.
- d. Other: I may entertain a volunteer cost saving time plan for the summer of 32 hours per week in order to work at the house more often; organize office files; Belinda will be submitting a request to you for tuition reimbursement for a class or two that she plans to take this summer or fall.

-

Deadlines

Reports: CSTE OH Indicators June 2008; Portal activity; Node meeting next week.

Time Management this week

Activity	Expected	Actual
EPHT	60%	80%
Child Lead	0%	0%
OH/ABLES	40%	20%
EOHP	5%	5%
Other	0%	0%

Alfred J. May, Jr.
 Comprehensive Health Planner/Epidemiologist
 Environmental and Occupational Health Program
 Maine Center for Disease Control and Prevention
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